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Community Health BOOK - For Revision purposes

health records and information technology (Kenya Medical Training College)

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CHAPTER ONE

1.0 INTRODUCTION TO COMMUNITY HEALTH

Expected Learning objectives

By the end of this session, the student will be able to:

a) Describe the concepts of community health;
b) Describe the community health historical trends in Kenya;
c) Explain the principles of community health;
d) Describe the factors that affect health status of a community;
e) Explain the roles of the health team and various health workers in delivery of community health;
f) Discuss MDG's and their relationship to health of the community.

1.1 Community Health Concepts

1.1.1 Introduction

Community health is concerned with prevention of disease, promotion of health and prolongation of life.

The aim of community health is to generate organized community effort to address public health problems and needs. In order to help the community prevent disease and improve their health, we need to understand how it organises itself as individuals, families and within the larger community.

Community health considers all the significant influences that affect an individual, family and the community. What is a family and community and what role does the individual play in community health?

Individuals
The individual shares behavioural characteristics with other people who are part of a particular culture. Culture is all those things which people learn, share and pass on to later generations. One of the most important of these things is language, and there are many languages in Africa. It's important to learn how to say the local greetings and to show respect and understanding for the culture of the community you serve.

Family
A family comprises of two or more individuals who are involved in a continuous living arrangement, usually residing in the same household and experiencing common emotional bonds. They also share certain obligations towards each other. A family has also been defined based on the following approaches:

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- Biological approach : a unit with biologic functions e.g. reproduction, perpetuating species;

- Psychological approach: a basic unit that influences personality, development and members give emotional and psychological support to each other (family support systems);

- Economic approach – members contribute to economic performance of the family in order to improve standards of living.

- Family as a system: a family can also be described as a living social system with a small group of interrelated and interdependent individuals who are organized into a single unit. The system and its parts have both structural and functional components. Structure refers to the arrangement and organization or the characteristics of the individual members who comprise the family e.g. age, gender, number of individuals in the family. Function refers to the purposes or goals of the family.

- Family as an open system: In an open system, the family tends to exchange materials, resources and information with the environment. The family constantly interacts with physical, social and cultural environment. A family functionality is therefore affected by what is happening in society.

Role of family in prevention of illness

As a basic unit of the society, the family represents the cornerstone of community health. The family provides children with basic education in language, beliefs and customs, in addition to food, clothing and shelter. Within the family you can find both the causes and solutions to their health problems.

Specifically the family can play the following roles:

- Prevention of infection: Environment sanitation, provision of safe water, proper housing, utilization of immunization services;
- Good nutrition: Kitchen gardens, income generating activities to improve on socio-economic status.
- Utilization of health service e.g. immunization, seeking medical attention in case of a sick member;
- Health education ⇔ New ideas and knowledge disseminated and shared among family members;
- Psychological support – counselling and support for each other.

Definition of a Community

A community is a group of people who live in a particular place such as a village, and who have a common interest in what happens there. It is a group of people with the following characteristics:

- Have common interests and goals;
- Live in one geographical area;

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- Share same resources such as water, health services;
- Have common beliefs, values, practices, norms and traditions;
- They interact and interrelate with one another (interdependence);
- They have a common leadership structure (social institutions).

A community is made up of a number of essential subsystems. These subsystems interact and interrelate continuously to bring about the overall functioning of the community. These subsystems also influence the health status of the people and include the following:

1. The socio-cultural of the system

This system influences the lifestyles of the community members, their priorities, their attitudes and values towards health and illness. Some cultural practices promote health while others impede health.

2. The political system (Leadership)

This is the system that has the responsibility of providing leadership in the development of health and health related laws and policies.

3. The economic system

The government's ability to provide health and other services to its citizens depends on the state of the economy. The poorer the economy, the more the disadvantaged the people will be. Low economic status is a leading cause of malnutrition and communicable diseases.

4. The education system.

Education is the main tool of changing behaviour and improving individual and community health. Low educational status perpetuates underdevelopment, harmful traditions and superstitions.

5. The religious system.

Religious system promotes good health, for example, when they forbid smoking, alcohol consumption, premarital and extramarital sex. However, some religions cause ill health by when, for example, they preach against seeking treatment in hospitals.

6. The environmental system

Environmental sanitation is an important indicator of public health. Clean water supply, proper disposal of waste and good housing are key to community wellness.

7. Communication and transport system.
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Communication includes all the means of contacting and exchanging information with one another such as roads, telephone, television, radio, internet, fax, etc. Communication system is important in disseminating health messages. Transport system helps people to move from place to place.

8. The health system.

The health care system exists to provide promotive, preventive, curative and rehabilitative services in hospitals, nursing homes, clinics, health centre, and dispensaries and through special health projects and programs.

In a rural community where everyone knows everyone else, there is a strong sense of belonging together. Tradition and public opinion is very important and people are careful to behave in a way that meets with community approval. You need to understand the community's traditions, the farming seasons and the type of foods they grow as all have an effect on health. You also need to know who the community leaders are and on what their authority rests.

1.2 Development of Community Health in Kenya

Before the coming of the colonialists to Kenya, there was a health service – the traditional healer, who is still with us today and plays a significant role in delivery of health care in our society.

Modern medicine came to Kenya initially through missionaries, then the Imperial British East Africa Company (IBEAC) and later through the government. There were disease epidemics even before the coming of colonial powers. For instance, there were small pox epidemics in 1880, 1898 and 1900. That was followed by severe outbreaks in Mombasa and Nairobi between 1901 and 1902. Thereafter, there were yearly epidemics till 1912.

In 1913, just before the First World War, Professor William Simpson form London School of Tropical Medicine was commissioned to visit Kenya and report on the sanitary conditions of the country. He recommended that there should be a clear distinction between curative and preventive services, a bigger budget for sanitation and the appointment of a Chief Sanitarian, now Chief Environmental Officer.

At a meeting of Provincial Commissioners in 1919, it was acknowledged that there was need to extend medical facilities to native rural areas and they recommended that dispensaries be built.

In 1920's, the colonial government started the training of local dressers, three for every 50,000 people. The dressers were trained to work with local native councils (now county councils) in the dispensaries already built in those areas. The first Public Health Act was published on the 6th September 1921 followed by 13 other acts.

The outbreak of the 2nd World War in 1939 affected health development in 1940's. Though the health services had a reasonable capacity to cope with war emergencies, the 280,000 African Soldiers who survived the war brought with them new diseases from wherever they had been. In 1942, Dr.

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Paterson, the then Director of Medical Services, wanted the service reorganized and advocated the need for research, expanded social services and improvement in housing and health care for the local people.

After independence, Kenya has recorded two phases in its health development. There was the phase of improvement (1960-1992) during which the goal of government was the provision of free health care for all, especially infants, children and mothers. The phase of degeneration (1993-2000) was characterized by declining life expectancy at birth, rising infant mortality rate, rising under five mortality, high maternal mortality and rising population estimated at 30.4 million in 2001 and projected at 35 million in 2008.

Since independence in 1963, Kenya has carried out various reforms aimed at promoting coverage of and access to modern health care in an attempt to attain its long term objectives of health for all. These reforms include decentralization of health delivery system, endorsing the principles of Primary Health Care as laid out in Alma Ata Declaration of 1978, and cost-sharing programme among others. The objectives of decentralization were to improve management, efficiency, accountability and responsiveness of health services. Kenya decentralized the health care system through the restructuring and strengthening of the district level. This was carried out under the District Focus for Rural Development Strategy, which started in 1983. The decentralisation programme created health management teams and boards at the district level in 1992, to represent community interest in health planning and coordinate and monitor the implementation of projects at district level. The programme granted autonomy to Kenyatta National Hospital and Moi teaching and Referral Hospital.

The cost–sharing programme was mooted in the 1984/1988 Development plan and its implementation started in December 1989 through the August Government Cabinet Paper. The government policy on cost-sharing was aimed at establishing and sustaining high quality health services.

Specifically, it was aimed to improved effectiveness and efficiency of health programmes, generation of more revenue for the health sector, improving the quality of health care, improving equity in the health delivery system and controlling the expenditure in the public sector spending on curative care. The main objective of the policy was to encourage increased cost recovery from users of public health facilities as one of the ways of involving additional resources.

In 1994, the Kenyan Health Policy Framework was launched articulating the government's commitment to improve the health of the population. The policy objective was to decentralize the management, provision and financing of health care by transforming the role of Ministry of Health from that of provider of service to that of policy maker and regulator of health care service provision. Future investments were to be used to encourage individuals and communities to play a more active role in preventing diseases and ill health and also to stimulate competition among providers of health care in both governments, private and mission sectors. To operationalise the document, the Ministry of Health developed the Kenya Health Policy Framework Implementation and Action Plan and established the Health Sector Reform Secretariat (HSRS) in 1996/97, to spearhead and oversee the implementation process. These policy initiatives aimed at responding to the following constraints; decline in health sector expenditure, inefficient utilization of resources, centralized decision making,

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inequitable resource allocation, inadequate management, poor information systems, outdated health laws, inadequate managerial skills at district level, worsening poverty levels, increasing burden of disease and rapid population growth. To implement the reforms, the National Health Sector Strategic Plan (NHSSP 1999-2004) that translated the policy objectives into programmes and activities were formulated.

Currently, the goal of second national health sector strategic plan (NHSSP 2005-2010) is to contribute to the reduction of health inequalities and to reverse the downward trend in health related impact and outcome indicators. It has introduced the The Kenyan Essential Package for Health (KEPH), which focuses on the health needs of the individual through the six life cycles of human development. Its Interventions emphasize community based care to be provided by community owned resource persons (CORPS). The indicators, targets and outputs of NHSSP II are used as the basis for the development of the Annual Operational Plans (AOP), which are prepared every year by each district.

1.3 Determinants of Health in Human Populations

Health is a state of complete physical, mental, social and spiritual wellbeing and not merely the absence of disease or infirmity (WHO).

The determinants of health refer to those factors and influences that shape or determine the health of individuals in a community. Some of the factors that determine health status of individuals, families and community include the following:

Socio-economic factors:

This concerns the economic development or growth found in a society. It focuses on the per capita income found within a population. It determines how poor or rich an individual, family or community is. High income determines living conditions such as safe housing and ability to buy sufficient and good food. There is strong evidence that higher social and economic status in associated with better health.

The Indicators of economic growth include:

- *Employment* – In most countries, employment is usually available in larger urban areas than in smaller towns and rural areas. Those employed can afford access to health services while the unemployed will be unable to access due to poverty. Rural-urban migration leads to overcrowding and congestion of available space. Consequently, slums areas are accompanied by health problems due to poor sanitation, poor living standards and poor access roads in case of emergencies. Some forms of employment also expose workers to health risks such as dust from quarries, textile and milling industries.
- *Budget* – The government budget for health services influences the health of communities. If the government allocates more money, it becomes possible to begin programmes aimed at preventive and promotive services.

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- *Education and literacy* – Development and growth of a country can be measured by the education level of its citizens. High literacy and education leads to better health. This is because education equips people with the knowledge and skills for problem solving and helps to provide sense of control over life issues. It also increases job opportunities and income security.
- I*ndustrialization* – Economic development of a country may be measured on its industrialization status. The more industrialized a country or a region is, the more developed it may be. Industrialization also comes with its own pressures and health problems e.g. air, land and water pollution, industrial emissions that contribute to global warming.

Genetic and biological factors

The basic biology and organic make-up of the human body are fundamental determinants of health. A number of diseases are passed down in families through genetic traits e.g. Sickle Cell Disease, Hemophilia, Mongolism (Down's syndrome), Diabetes, some forms of Asthma.
A woman may inherit a physique that predisposes her to Cephalo-Pelvic Disproportion (CPD) during pregnancy leading to obstructed labour hence necessitating delivery through caesarean section.

Demographic factors

Demography is the study of human population and its characteristics are:
- Changes in population size;
- Age and sex distribution;
- Geographical distribution;
- Increase in mortality and morbidity rates;
- Vital statistics – Birth rates, death rates, marriage, divorce, immigration, outmigration data.

Geographical factors

- Climate – is related to temperature, sunlight, wind, rainfall, humidity etc. Different climatic conditions are associated with different health conditions/diseases.
- Land and soil type – the availability of land and type of soil determine the agricultural activities that a population is involved in. Food production and nutrition are determined by these factors of land and soil. Poor food production may lead to malnutrition due to food shortage or food that is poor in nutrients.
- Rivers and lakes (Water bodies) – influence health by providing food such as fish and water supply for domestic and farming use. Water may also increase health risks through the spread of faecal-oral diseases such as typhoid and dysentery if contaminated. It may also contribute to vector breeding such as mosquitoes and increase malaria.

Culture

People's cultural beliefs can influence their attitude towards something and hence their behaviour. Health seeking behavior is strongly related to cultural beliefs, for example, if a community believes that an illness is caused by witchcraft they may not visit the health centre. Also cultural practices

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determine the kind of food people eat and don't eat (food taboos), and encourage extended family responsibilities, family support and good mental health especially in time of crises.

Environmental factors.
These are the potentially hazardous agents or factors in the environment.

There are three types of environments, namely:

Physical environment is an important determinant of health and includes:

- *Water* is essential for control of diseases that are waterborne or water related e.g. typhoid and cholera. In is therefore necessary to provide safe and adequate water to everyone.
- *Air*: exposure to polluted air can lead to serious respiratory diseases such as bronchitis and lung cancer.
- *Radiation* – exposure to radiation especially from x-rays may cause diseases such as cancer of the skin or blood.
- *Housing and planning* – poor housing and infrastructure lead to overcrowded living areas, poor sanitation and poor transportation which can influence health negatively.
- *Heat, light and noise* – these can lead to fires, accidents and disabilities of hearing.

Hi

Biological environment
This one is made up of all living things: people, vegetation, infective organisms such as amoebae, bilharzia, insects such as mosquitoes, and animals including stock and predators.

Social environment
This is concerned with support from families, friends and society which is associated with better health (social interaction and social support networks). Man is a social being and his interaction with other people can contribute positively or negatively to health.

Lifestyle/Personal factors.

Many health problems are attributable to personal choices of life e.g. alcoholism, drug abuse, unsafe sex leading to STI's and HIV/AIDS, sedentary life that increases risk of obesity and heart disease.

Political factors

The leadership of a country influences the health of its citizens through budget, planning and provision of health services to the people. National social health insurance enables everybody to access health care. Distribution of resources based on political affiliations may affect people's health. The government as a policy maker may permit a project to go on for economic reasons but may later affect the health of people living nearby, for example, the Webuye paper mills, Tiomin mining in Kwale.

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Health Care System.

Health services, particularly those designed to promote health, prevent diseases and restore health contribute to the health status of the population. Access to health care services, affordability, availability and the quality of health services are critical issues in health care delivery.

1.4 Principles of Community Health Practice

The following are the principles of community health practice:

- Community health encourages ownership by the community. The community participates, the community becomes a partner and the community eventually owns the programme. In other words, there is genuine partnership of health workers and community members.
- Community health responds to the needs of the people. The health worker starts with the people, helps them identify their problems and needs through community diagnosis and works with them in finding solutions to the problems and needs.
- Community health encourages self-reliance. It aims to bring about health, self-reliant communities. Through health education by the community health workers, community becomes informed and empowered with knowledge, skills and positive attitudes (community empowerment).
- Most of the health care interventions are undertaken at the peripheral level where people live (hence the term Peripheral Health Care). Health services are taken to people's homes or accessibility of the care is of vital importance in community health.
- Community health services should be available to all the people and at a cost they can afford. A person's health needs rather than the socio-economic status determines the type of health care received. There is no discrimination in the provision of services.
- Community health services follow a comprehensive and integrated model. There is integration of curative, preventive, promotive and rehabilitative services.
- The health services offered in community health rely heavily on the available community resources in order to achieve self-reliance and sustainability.
- In community health, the health care activities are fully integrated with the activities of other sectors involved in community development such as agriculture, education, public work, housing, transport and communication. Community health programmes work in partnership with others whenever possible and this is what is called intersectoral or multisectoral collaboration. There is need for health workers to realize that although we can do little by ourselves, we can do a great deal when we work together with others.

1.5 Role of Various Health Workers in the District Health Team

As we mentioned earlier, the district is the key management level for health care services. The district health service is headed b a district medica tol officer or medical officer of health. He heads the district health management team which includes the following staff:

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District Medical Officer.

He or she is responsible for not only the health center but also for health of the whole district. As the leader of the District health team, he works closely with the team and provides a link with other administrative officers in the district such as Education officer, Agricultural officer, Veterinary officer and other non-governmental organizations offering health care in the district. His work includes:

- The general administration of the entire district;
- The control and prevention of communicable diseases in the district by organizing immunization campaigns, health education, investigation and control of epidemics and the collection of data on the disease burden of the district;
- The monitoring of health of people working in factories and other industrial places. Here, he operates with the local labour officer and the Inspector of factories;
- The provision of school health services;
- Administration of medical stores, equipment and transport;
- The writing of Annual Report on the state of health in the district from the records kept, so that he can plan his future services more efficiently.

The District Public Health Nurse

He/she is responsible for the coordination of all nursing services and supervises all nurses in the district. The DPHN also ensures all records kept by the nurses are up to date; and that the standard of nursing practice is high. He/she organizes seminars, workshops and in-service training or refresher courses, particularly when new methods and techniques are to be introduced in practice. Other responsibilities include ensuring the supply of drugs, vaccines and other equipment to the health sectors and compiling Annual Report on state of Nursing Services in the District from the records kept by Nurses.

The District Public Health Officer

He or she is also a member of the DHMT and supervises the work of other public health officers and technicians. The DPHO oversees the implementation of environment hygiene and sanitation programmes .e.g. refuse disposal, sewage systems, and protection of water supplies, improved housing and inspection of food factories, market places and eating places. He or she is also responsible for mass disease control campaigns and organises community meetings (barazas) to discuss health matters with the community.

The Clinical Officer

He or she is usually in charge of the health center. The responsibilities include provision of curative services and referral of difficult cases to the provincial level. He or she is also the Secretary to the Health Centre Board of Management meetings and chairs meetings of the health centre staff where they discuss management issues.

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1.6 The Millennium Development Goals (MDG's)

What are the Millennium Development Goals (MDG's)?
At the Millennium Summit in 2000, representatives from 189 countries committed themselves to a world in which sustaining development and elimination of poverty would receive the highest priority on the global agenda. The Millennium Development Goals (MDG's) summarizes these commitments and have been commonly accepted as a frame work for measuring development progress. They are an integral part of the Road map towards the implementation of the United Nations Millennium Declaration.

There are Eight (8) Millennium Development Goals (MDG's), namely:

Goal 1 Eradicate extreme poverty and hunger

- Have, between 1990 and 2015, the proportion of people whose income is less than $1 a day.
- Have, between 1990 and 2015, the proportion of people who suffer from hunger.

Goal 2 Achieve universal primary education

- Ensure that, by 2015, children everywhere, boys and girls alike, will be able to complete a full course of primary schooling.

Goal 3 Promote gender equality and empower women

- Eliminate gender disparity in primary and secondary education preferably by 2005 and in all levels of education not later than 2015.

Goal 4 Reduce child mortality

- Reduce two-thirds, between 1990 and 2015, the under five mortality rate.

Goal 5 Improve maternal health

- Reduce by three-quarters, between 1990 and 2015, the maternal mortality ratio.

Goal 6 Combat HIV/AIDS, malaria and other diseases

- Have halted by 2015 and begin to reverse the spread of HIV/AIDS.
- Have halted by 2015 and begin to reverse the incidence of malaria and other major diseases.

Goal 7 Ensure environment sustainability

- Integrate the principles of sustainable development into country policies and program and reverse the loss of environment resources.
- Have, by 2015, the proportion of people without sustainable access to safe drinking water.
- Have achieved, by 2020, a significant improvements in the lives of at least 100 million slum dwellers

Goal 8 Develop a global partnership for development

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- Develop further an open, rule-based, predictable, non-discriminatory trading and financial system (includes a commitment to good governance, development and poverty reduction ⇔ both nationally and internationally).

How do the Millenium Development Goals link with the global health agenda?

The MDG⇔s give high prominence to health. Three of the eight MDG's, that is, reducing child mortality, improving maternal health and combating HIV/AIDS, malaria other diseases are directly related to health. Overall, the MDG⇔s provide a set of outcomes that are relevant to the development of national health policy frameworks such as poverty reduction strategies and national health policies focusing on the poor and for tracking the performance of health programmes and systems.

Kenya is signatory to the MDGs and is committed to their achievement. As part of these initiatives, Kenya has translated these international targets, set by the MDG into national targets and further translated them into regional and district level targets as part of the next year Annual Operational Plans, so that they can inform and guide local priority setting and resource allocation.

Review questions

1. Discuss the concepts of community health;
2. What is the community health historical trends in Kenya;
3. State the principles of community health;
4. Identify the factors that affect health status of a community;
5. What is MDG's and their relationship to health of the community?

CHAPTER TWO

2.0 Primary Health Care

Learning objectives

By the end of this session, the student should be able to:

a) Explain the concepts and background of rural health, primary health care and community based health care.
b) Describe the Principles, pillars and strategies of Primary Health Care.
c) Describe the elements of Primary health Care and their implementation in Kenya;
d) Describe how to organise PHC activities in relief programs;
e) Discuss community financing in primary health care;
f) Explain the challenges faced in PHC implementation.

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2.1 Concepts and Background of PHC in Kenya

The development and organization of health systems based on primary health care concept was stimulated by three important events of which Kenya was a signatory. These were

1) The observation by the 1977 World Health Assembly that there were a lot of injustices and inequalities in the distribution of health resources in many parts of the world (particularly in the developing world).
2) Adoption of the Alma Ata Declaration in 1978 of Health for All by the year 2000 by all WHO member countries.
3) Adoption in the same Alma Ata Conference of Primary Health Care strategy as the means of achieving the goal of Health for all by year 2000.

In the 1977, World Health Assembly, the Government of Kenya along with other member states of WHO, endorsed the worldwide social objective of "The attainment by all people of the world by the year 2000 of ay level of health that will permit them to lead a socially and economically productive life *(WHO, 1977*).

However, many countries in the developing world recognised the fact that it was not possible in the foreseeable future for them to achieve this worldwide social objective. This was because many lacked the resources needed to develop and run health services. They needed to adopt a strategy that allowed them to use the available resources to give some benefit to everyone and provide special attention to those at high risk. So, the member Governments endorsed the Primary Health Care strategy for the provision of health services for all.

The strategy for the implementation of PHC was adopted by the Kenya Government to provide health services to its population, majority (80%) of who live in the rural areas.

2.1.1 The Definition of Primary Health Care

In 1978, the Alma Ata international conference on primary health care defined primary health as:

> .. essential health care based on practical, scientifically sound and socially acceptable methods and technology, made universally accessible to individuals and families in the community through their full participation, and at a cost that the community and country can afford to maintain at every stage of their development in the spirit of self-reliance and self-determination. In addition, it forms an integral part both of the country's health system, of which it is the central function and main focus, and of the overall social and economic development of the community. It is the first level of contact of individuals, the family and the community with the national health system, bringing health care as close as possible to where people live and work.

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PHC is not only:

- Primary medical care
- First contact medical or health care
- Health services for all

PHC is and does the following:

- It is intended to reach everybody, particularly those in greatest need
- It is intended to reach the home and family level, and not to be limited to health facilities
- It is intended to involve a continuing relationship with persons and families

Preventive and Promotive Health Care (PHC) includes the basic clinical, preventive, and promotive health services that should be readily accessible to all members of the population. Emphasis is on improving family health, with particular focus on mothers and children; increasing coverage and accessibility of essential health services; improving the quality of services; and pursuing an integrated inter-sectoral and multi-disciplinary approach with community participation in the planning, delivery, and monitoring of health services

Key statements which identify PHC as essential health care are:

- PHC is universally accessible to individuals and families in the community;
- PHC is socially acceptable to all, meaning that the health care is appropriate and adequate in quality to satisfy the health needs of people, and is provided by methods acceptable to them within their social cultural norms;
- PHC is affordable, that is, whatever methods of payment used, the services should be at a price the community can afford;
- PHC promotes full participation of individual, families and communities;
- PHC is appropriate technology i.e. the use of methods and technology which use locally available supplies and equipments.

2.1.2 The Concept of Primary Health Care

A concept is an idea about an issue, event or practice.

So, the concept of PHC can be described as ideas about the implementation of health care for all.

It is easily identified using the 5 A's which describe its attributes as follows:

- Accessible — That is, the services are geographically, financially and culturally within easy reach to the whole community.

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- Acceptable — The quality of health services offered are appropriate, adequate, and able to satisfy the health needs of people; and are provided by methods which are within their social cultural norms.

- Affordable — That the services are provided at a cost that the community can afford.

- Available — The health structures and services are easily available to the community members and they also help them to assume responsibility in promoting their own health.

- Appropriate Technology: Utilizing existing methods, techniques, and resources within the community.

2.1.3 The Seven Pillars of PHC

Primary health care is a strategy of health care delivery which creates a partnership between the consumer of the health services and health care professionals. They both actively participate in the achievement of the common goal of improved health.

P*rimary health care is founded on 7 pillars as follows, health system, priority, science, culture, equity, participation and sustainability.*

2.1.4 Fundamentals of PHC

Fundamentals of PHC are basic rules or beliefs that are essential to the existence, development or success of primary health care.

The following are the fundamentals of PHC:

- PHC reflects and evolves from the economic conditions and socio-cultural and political characteristics of the country and the communities; and is based on the application of the relevant results of social, biomedical and health services research and public health experience.

- PHC addresses the main health problems in the community, providing promotive, preventive, curative and rehabilitative services accordingly.

- PHC includes, at least: education concerning prevailing health problems and the methods of preventing and controlling them; an adequate supply of safe water and basic sanitation; maternal and child care including family planning; immunization against the major infectious diseases; prevention and control of locally endemic diseases; appropriate treatment of common diseases and injuries and provision of essential drugs.

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- PHC involves, in addition to the health sector, other sectors such as agriculture, animal husbandry, food industry, education, housing, public works, communication and other sectors. It demands the coordinated efforts of all these sectors.

- PHC requires and promotes maximum community and individual self- reliance and participation in the planning, organization, operation and control of health services, making fullest use of local, national and other available resources. Through appropriate education the communities themselves are empowered to participate.

- PHC should be sustained by integral, functional and mutually supportive referral systems leading to the progressive improvement of comprehensive health care for all, and giving priority to those most in need.

- At local and referral levels, PHC relies on health workers, including physicians, nurses, midwives, auxiliaries and community workers, as well as traditional practitioners who are suitably trained both socially and technically, to work as a health team and to respond to the expressed health needs of the community.

2.2 Elements of PHC

In the Alma Ata conference of 1978, eight essential elements of PHC were identified. However, individual countries were given the liberty to add any other elements they felt were relevant to their own country. Kenya has added 6 other elements.

The PHC elements listed at the Alma-Ata Declaration were as follows:

1. Education concerning prevailing health problems and the methods of preventing and controlling them.

2. Local disease control

3. Expanded programme of immunization

4. Maternal and child health care and family planning

5. Essential drug supply

6. Nutrition and adequate food supply

7. Treatment and prevention of common diseases and injuries.

8. Safe water supply and good sanitation.

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Kenya Government has added more elements to the ones we have just discussed. These are:

Health Education

Health education is education that is intended to have a positive impact on health. It is a process of dialogue with community members to find out appropriate responses to health problems as well as empower them with the knowledge and insight they need to understand how their behaviour affects their health. Health education today has extended its scope beyond disease prevention and control to health promotion. It gives individuals and communities the incentive to promote the conditions that maintain good health.

Therefore as you can see, health education is an integral part of all health services and all health personnel including yourself have an important role to play in organizing appropriate health educational programmes at all levels in the community.

Promotion of food supply and proper nutrition

As you are well aware, nutritional deficiency states are particularly noticeable among pregnant and lactating mothers, infants and children. This may be due to the prevailing cultural or economic factors in the community. As a community health nurse, it is your responsibility to take suitable measures to prevent and treat diarrhoea diseases, intestinal parasites and other diseases which lead to nutritional deficiency states. It is also your responsibility to support health promotional measures such as child spacing, nutrition education, kitchen garden and food hygiene. In co-ordination with other sectors, you should also encourage community members to grow more foods, prevent post-harvest spoilage through construction of simple food stores, and to keep poultry and dairy cattle.

Water supply and basic sanitation

Safe water and sanitation is not available to a major section of our population, yet, it is essential for life. Many water borne diseases which are prevalent in the community can be prevented if communities gain access to safe water and adopt proper refuse and faecal disposal.

So under this element, effort are made to bring together the different factors from related sectors to survey and identify sources of safe water and carry out proper analysis of the water. At the same time, community health workers should educate community members on how to protect wells and springs from contamination, how to construct latrines, compositing facilities and soakage pits.

Maternal and Child Health and Family Planning

Children make up one-half of the community and their mothers another fifth. So on numbers alone, health care for mothers and children forms the greater part of community health. Also, mothers and children run a great risk of injury and disease because their lives are concerned with beginnings and growth. MCH/FP services are therefore aimed at promoting the health of mothers and children by

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reducing the maternal and child mortality rates and enabling women of childbearing age to have the desired number of pregnancies and at the right interval. MCH/FP care has the following four main functions:

- Antenatal care
- Intranatal
- Prenatal care
- Postnatal care
- Family planning

Since 1980, the issue of family planning has gained momentum and highlighted an issue that was formally regarded as unimportant. In response, our Government established the National Council for Population and Development to coordinate all population and family planning activities. It also set up the service component of the family planning program within the ministry of health. Also, NGOs such as FPAK, CHAK, and the Catholic Secretariat play an important role in both motivation and service provision.

The practice of family planning is an old African tradition. What is new is the variety of methods which have been introduced to prevent or delay pregnancy?

Immunisation

Kenya has for some time now implemented immunization activities through the Kenya Expanded Programme on Immunization (KEPI). Immunization is a very effective means of primary prevention against certain endemic and epidemic diseases. Kenya has a long history of immunization programmes. Health workers have been trained on how to motivate and encourage mothers to bring their children for immunisation, as well as how to identify suspected cases of immunizable diseases such as, measles, poliomyelitis and neonatal tetanus, using standardized case definition (disease surveillance).

Local disease control

There are many endemic diseases in this country, some of which are confined to particular areas.

Examples of endemic diseases in this country include the following:

Malaria Control

Each district in Kenya is required to determine malaria endemicity and plan and implement an appropriate control strategy. The other factor to be considered is the objective of control activities. Four levels of control are possible as described below.

Level 1
The objective is to prevent malaria mortality and hence the control strategy is to make chemotherapy available as close as possible to every family.

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Level 2
The objective is to control mortality and morbidity due to malaria. The control strategy here involves chemotherapy as close to each family as possible and chemoprophylaxis with simple anti - mosquito measures.

Level 3
The objective is to control malaria mortality, morbidity and prevalence. This requires chemotherapy and chemoprophylaxis with effective mosquito control.

Level 4
The objective here is to eradicate malaria completely.

Given the economic situation in Kenya, the health infrastructure and the complexity of the logistical demand of the methods of control mentioned above, the first priority of malaria control for us has to be that of mortality and morbidity particularly in areas of unstable malaria.

Treatment and prevention of common diseases and injuries

Curative care is important in its own right as it provides a powerful mechanism for teaching preventive and promotive care.

The list includes:

- Diarrhoea diseases
- Skin diseases
- Worm infection
- Common accidents requiring first aid;
    - burns
    - wounds
    - bites and stings
    - allergic shock
- Eye conditions
- Acute respiratory infections

Supply of Essential Drugs

Essential drugs are basic drugs used to treat minor ailments or conditions at the dispensary and health centre levels.

Kenya has been a pioneer in the establishment of an effective drug kits system which regularly delivers drugs to health units. Community pharmacies have also been established in remote rural areas to improve access to drugs in the community. As a community health nurse you have a major

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responsibility in ensuring that patients have access to essential drugs and know how to manage their drug regimens for optimal effect.

Mental Health

Mental health services should not be viewed in isolation but as an integral part of the other services that are needed to achieve the complete health of individuals, families and communities. Therefore health workers should:

- Be oriented to look at mental health as part and parcel of PHC;
- Promote good mental health practices through health education of the family and community in order to create awareness;
- Provide facilities in all health institutions and service delivery points for education, detection, treatment or referral of mental health problems.

Dental Health

Dental health is a strategy of care focusing on the promotive and preventive care of teeth and the oral cavity. I am sure from your experience you are aware that dental diseases are one of the most widespread diseases in our communities and yet they are largely preventable. The ministry of health has established fully fledged dental care units in all health facilities.

Community Based Rehabilitation

Many developing countries such as Kenya included this element in order to give special attention to the management and prevention of disabilities arising from congenital defects, chronic non-communicable diseases such as cancers, and accidental injuries.
Rehabilitation services are now being integrated at all levels of health care delivery including at the family and community level.

HIV/AIDS Prevention

The Kenyan Government has set out technical and ethical approaches aimed at meeting the challenges presented by the HIV/AIDS pandemic. These include:

- Adequate and equitable provision of health care to the growing numbers of HIV infected people falling sick;
- Treatment of other sexual transmitted diseases that increase peoples biological vulnerability to HIV infection;
- Reduction of women's vulnerability to HIV infection by improving their health, education, legal status and economic prospects;
- A supportive socio-economic environment for HIV/AIDS prevention.

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The PHC approach emphasises the need to involve individuals, family members, and community members in the prevention and control of HIV/AIDS.

2.3 Principles of Primary Health Care

In the implementation of the Primary Health Care elements which we have just discussed, a number of principles are involved. Although the details vary from country to country each principle must be considered during the implementation of PHC.

A principle is a rule or basic belief that has a major influence on the way in which something is done. Therefore principles of PHC are rules or guidelines that govern the implementation of PHC activities. There are five basic principles which govern the implementation of PHC.

These are:

Equitable distribution

Equity is the fair and reasonable distribution of available resources to all individuals and families so that they can meet their fundamental and basic needs. Services should be physically, socially, and financially accessible to everyone. People with similar needs should have equal access to similar health services. To ensure equal access, the distribution of resources and coverage of primary health care services should be greatest in those areas with the greatest need. This principle should be taken into account when deciding on the location of new health facilities, outreach services points, or during introduction of new health programmes, especially those that require payment for services.

Manpower Development

We know that primary health care aims at mobilizing the human potential of the entire community by making use of available resources. This principle facilitates the identification and deployment of the necessary health personnel as well as the training and development of new categories of health workers to serve the community.

Comprehensive PHC requires health workers to identify solutions that involve the community, as follows:

- It is not enough to provide oral rehydration solution and medical treatment to a sick child with
- Diarrhoea. Maintaining the health of the child also requires providing family education on child care and environmental hygiene, as well as improving access to food.
- In addition to counselling on breast-feeding, growth monitoring, nutrition rehabilitation, and child care, a nutrition program should promote weaning foods that are available locally.
- PHC services for healthy people (e.g., pre-natal care, immunisation, health education) should be established as soon as possible through community-based health interventions.

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Community participation

As you learnt in Unit 1 Section One of this Module, community participation is the process by which individuals, families and communities assume responsibility in promoting their own health and welfare. The PHC strategy underlines the importance of full community participation, especially in health decision-making. Community members and health providers need to work together in partnership to seek solutions to the complex health problems facing communities today. In addition to the health sector, families and communities need to get actively involved in taking care of their own health.

Communities should participate in the following:

- creating and preserving a healthy environment
- maintaining preventive and promotive health activities
- sharing information about their needs and wants with higher authorities
- implementing health care priorities and managing clinics and hospitals

Appropriate technology

Appropriate technology is the kind of technology that is scientifically or technically sound and adaptable to local needs, and which the community can afford to maintain at every stage of their development in the spirit of self-reliance and self-determination. It includes issues of costs and affordability of services, type of equipments and their pattern of distribution throughout the community. Care-givers should be trained to deliver services using the most appropriate and cost-effective methods and equipment for their level of care.

Multi-sectoral or Intersectoral approach

PHC requires a co-ordinated effort with other *health-related sectors* whose activities impact on health e.g., agriculture, water and sanitation, transportation, education, etc. This is necessary to achieve social and economic development of a population. The health sector should lead this effort. The commitment of all sectors may increase if the purpose for joint action and the role of each sector is made clear to all concerned.

Lessons drawn from past experience clearly indicate that the health sector cannot achieve much in isolation. It must work in close collaboration with other sectors in the community in order to succeed in promoting the community's health and self-reliance.

Often, the health sector works in collaboration with the following sectors:

- Agriculture;
- Water and sanitation;
- Animal husbandry;

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- Education;
- Housing;
- Public works ;
- Transport and communication;
- Roads and housing;
- Reclamation, development of arid and semi arid wastelands;

These sectors need to co-ordinate their plans and activities in order to contribute towards the health of the community and avoid conflicts or duplication of efforts.

2.4 Implementation of PHC Elements

Overview of Health Services in Kenya before Implementation of PHC.

Since Kenya became independent in 1963, our Government has all a long demonstrated its commitment towards the provision of quality health services for its people. In its various manifestos and development plans, it has identified health as one of the basic needs and an essential precondition for the overall economic development and social progress of this country.
The major milestones achieved by our Government in health care development are captured in the following chronology of events.

- In 1965 the Government introduced free medical treatment in Government medical facilities in line with the policy guidelines of the KANU Manifesto.

- In 1967 the National family planning programme was started.

- In 1970 the Central Government took over the running of health services from local councils.

- In 1971 – 1972, a joint GOK/WHO mission formulated the proposal for the improvement of rural health services in the country and establishment six Rural Health Training Centers (RHTCs). This was done in order to provide adequate health coverage to the rural population.

In 1984 a Community based Health Care unit was set up within the integrated Rural Health and family planning project.

Indeed, the concept of community participation in development activities is not new in Kenya. You will remember the introduction of the *Harambee* (self-help) movement which encouraged people to contribute their resources and participate in the development of healthcare and education. People's efforts were directed towards construction of physical facilities like classrooms and wards with the hope that the Government would take over their management. Similarly, community participation is an important cornerstone of the PHC strategy. The PHC strategy relies on the abounding spirit of self-help among community members and endeavours to empower them to improve their health.

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Having seen where we have come from, let us now look at direction that our health care delivery services are taking. This can be informed by looking at some of the key health policies that have influenced health development.

Major Health Policies Guiding Current Health Development Plan

The steady development of PHC has necessitated a continuous review of existing policies in the health sector.

This policy was introduced by the Government in July 1985 to decentralize decision making to the grass roots and turn the district into a centre for the planning and implementation of Government projects.

As a result of this strategy, the management capabilities of health personnel at the district level were strengthened thus reducing many challenges which they experienced before.

The role of the District Health Management Teams was strengthened in line with district focus for rural development strategy.

- Increasing coverage and accessibility of health services in rural area.

It was realized that development of the rural health infrastructure had lagged behind because of financial constraints. Yet experience had shown that preventive and promotive health programmes were more cost effective if adequately supported. So the Government made a deliberate effort to redirect capital from major capital projects to small-scale projects at the district and sub-district levels. Effort were also made to support preventive and promotive health programmes and further investment in the rural health infrastructure by improving service delivery methods and increasing the number and quality of trained health manpower.

- Consolidating urban/rural, curative, preventive and promotive services.

Here emphasis was put on training all health cadres in preventive and promotive methods. Personnel located at hospitals and other static facilities were encouraged to include health education as a routine component of PHC.

- Intersectoral collaboration

Intersectoral collaboration means working together with other sectors whose activities have a direct influence on health. Health is too important to be the responsibility of the health sector alone. As we mentioned earlier, other sectors whose activities have a direct influence on health include ministries of agriculture, water, housing, culture and social services, and so on.

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The current development plan has set out the following policies to guide health development:

- Increasing emphasis on MCH/FP services in order to reduce morbidity, mortality and fertility rates;
- Strengthening ministry of health management capabilities with an emphasis on the district level;
- Increasing inter-ministerial coordination;
- Increasing alternative financing mechanism for health care.

These major policy guidelines show the explicit direction in which health care delivery services in Kenya have taken. For instance, there has been a clear shift from the earlier policy which provided free medical services to one which has introduced cost sharing. The development of the Community Based Health Care (CBHC) approach as a basic component of PHC is another important milestone. This approach emphasizes community participation in environmental health activities, prevention of diseases, establishment of community health funds and income generating activities.

The Alma Ata conference set as its target Health for All by the Year 2000. Since then our Ministry of Health has reviewed and revised its strategies to follow the primary health care guidelines. Through the implementation of the policies we have just discussed, it has organized a number of healthcare activities within communities according to their needs and conditions.

While these efforts have led to an improvement in accessibility, availability, affordability and acceptability of health services, there are still considerable constraints and a lot more needs to be done to completely integrate PHC.

The year 2000 which was set as the target for the achievement of health for all came and passed. Even five years later we have not yet achieved health for all.

There are certain rational steps which we can adopt in order to effect the necessary changes at the community and location level.

- Training and retraining of health personnel, the community and community leaders using appropriate methodology and exposure of health personnel to the communities;
- Strengthening intersectoral collaboration at the community level;
- Good governance and greater commitment of our political leadership to the concepts of Primary Health Care;
- Intensification of community involvement and existing community initiatives for health and development through increased awareness;
- Extension of the existing health services infrastructure in support of PHC to remote areas through outreach programmes or creation of new health units.

Organization of Health Services in Kenya

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At Headquarter Level
Here the Ministry of Health (MOH) is responsible for setting policy, coordinating activities of Government and non-Governmental organizations, managing the implementation of policy changes regarding Government services such as user charges, and monitoring and evaluating the impact of policy changes.

At the provincial level, the roles of the Provincial Medical Officer of Health (PMOH) and members of the Provincial Health Management Team (PHMT) are to act as a strong intermediary between the central Ministry and districts and to oversee the implementation of health policy (maintenance of standards of quality, performance, coordination, regulation and control of all health services in the public and private sectors in their areas of jurisdiction).

The role of the PMOH and PHMT with regard to the cost sharing programme is to issue Authority to Incur Expenditure (AIEs), guide, monitor and supervise the District Medical Officers of Health (DMOHs) and facility managers in the province in the management of cost sharing activities. PMOs are ex-officio members of the District Health Management Boards (DHMBs) in their provinces. They receive copies of all minutes of DHMB meetings and all long-term plans for the district approved by the Board. The Boards should inform them of any suspected irregularities in the running of district health services.

At the district level, the DHMBs oversee all health sector activities and their functions are not limited to the management of cost sharing funds. The Government established DHMBs with representatives of consumers and other interested groups to ensure prudent use of such funds.

The Primary Health Care Committee

This is one of the committees under the District Health Management Team. The Primary Health Care Committee oversees all preventive and primary health care (P/PHC) activities in the district or hospital. Its specific responsibilities are to:

- Participate with the Primary Health Care (PHC) Core Team in developing annual PHC plans. This involves a review of services, assessment of needs and setting of priorities for P/PHC activities in the district/hospital.
- Ensure that the DHMT is submitting plans for the expenditure of 25% of cost sharing revenue and is spending the funds as planned.
- Receive reports on preventive, promotive, community-based and primary health care activities in the District (GoK and NGO) or hospital.
- Obtain annual reports of health statistics with epidemiological data and preventive measures being taken to address the major problems.
- Promote intersectoral collaboration on issues of sanitation, water, nutrition and health education.
- If applicable, review reports on Bamako Initiative activities.
- Receive periodic reports from the PHC Coordinator on activities being carried out in each of the 8 components of PHC and insist on setting targets for expanding coverage of these services and, thereafter, monitor the achievement of these targets.

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- Look into urban sanitation problems (e.g., inadequate refuse collection, unsafe water points, etc.), and promote clean-up and safe water supply operations where needed. Also receive complaints and recommend solutions.
- Work with the DHMT/HMT to establish contingency plans for epidemics and provide the necessary support during such epidemics.

How are PHC Elements Implemented At Different Levels of Health Care Provision?

Family Level

In unit 1 of this module we defined the family. Can you remember what we said?
We said that the family is the basic unit of social organization in a community. Being the basic unit in a community, the family is therefore the nucleus and main focus of each essential element of PHC. Let us briefly look at how each element is implemented at the family level.

- *Education*

The family does not only provide its members with food, clothing and shelter. It also gives its members basic education in language, beliefs and customs. As we mentioned in Unit 1 of this module, families have a strong influence on what each member does. Thus when we educate families on how to promote their health and prevent disease we impact positively on the health of the entire community. Education for the promotion of health and the prevention of disease is most effective in the home environment because it creates an ideal atmosphere for effective teaching. Demonstrations can be done in the home setting with the active participation of all family members.

- *Nutrition and food supply*

Good nutrition is a basic component of the healthy growth and development of each family member. In order to strengthen food and nutrition activities at the family level, you need to teach the members about good food and nutrition practices including appropriate methods of growing and storing food.

- *Water and sanitation*

Motivating the family to start the "three pots" system where applicable. They should also be trained on how to protect springs and wells; how to construct and use latrines; simple personal hygiene measures such as hand washing; and the use of rain-roof catchments to harvest water.

- *Maternal and child health and family planning*

As you well know, certain traditional practices in the family towards pregnancy, labour and puerperium can impact positively or negatively on the health of a mother and child. It is your responsibility to encourage good practices and discourage harmful ones. Families need to be educated on the importance of antenatal care, immunization and family planning. This can be done through community health workers and opinion leaders such as, teachers, religious leaders, social workers. We can also use model families in the community to reinforce our teachings.

- *Immunization*

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It is the responsibilities of families to take children for immunization. Your role as a health worker is to educate them on how to read the road to health card, the need for immunization and how it works. With the assistance of TBAs and CHWs, you should also assess and refer children in the homes for immunization.

- *Control Of Endemic Disease (Malaria)*

The main objective is to control mortality particularly in stable malaria areas. The role of the family here is to identify the nearest source of anti malarial treatment; determine the dosage for the treatment of various age groups in the family; manage fever by tepid sponging and know when to take the patient to the nearest health facility; continue feeding the sick person as normally as possible; determine where and when to seek help; recognize and use simple protection methods such as mosquito treated nets; ensure family members at risk of dying from malaria receive chemoprophylaxis; and clear bushes and stagnant water around the dwelling.

If the family implemented these simple measures, they would considerably reduce mortality due to malaria.

- *Treatment of Common Conditions*

Common conditions are diseases which tend to occur very often in the family. They include:

- Malnutrition
- Anaemia
- Malaria
- Diarrhoeal diseases
- Acute respiratory infection
- Worm infestation
- Schistosomiasis
- Scabies
- Conjunctivitis
- Otitis media

The role of the family here is to recognize the signs and symptoms of these conditions and seek help from the nearest health facility. In addition it is their responsibility to ensure the prescribed treatment is taken correctly and to fully support the patient until they fully recover. They should also understand the causes of these diseases and take the necessary measures to prevent them.

- *Mental Health*

The role of the family in the implementation of the mental health element is as follows:

- recognizing and accepting that mental health problems are like any other disease;
- seeking help as soon as abnormal behaviour is detected among any of the family members;
- adopting practices that promote good mental health, such as breast feeding and family support in times of crisis;

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- avoiding behaviour and practices that contribute to poor mental health such as over permissiveness and rejection of their young ones.

- *Dental care*

The family plays a very important role in the implementation of dental health care element. This is because the family is able to reinforce habits that lead to healthy teeth and gums, such as the use of local tooth sticks; eating of indigenous foods, regular dental checkups and avoiding consumption of large amounts of refined sugar.

- *Community based rehabilitation*

Family members should be educated and sensitised by CHWS on how they can reduce disability in the community. They should take their children for immunization to prevent diseases like polio; attend antenatal clinics for early detection and management of those with complicated pregnancies; participate in the care of members with rehabilitative needs; and seek support from organizations whenever necessary

- *HIV/Aids/TB Prevention*

Since families have a very strong influence on what each member does, they can achieve a lot in HIV/AIDS/TB prevention by encourage single sex partners; talking openly to their children about the importance preventing HIV/AIDS; nursing their members with HIV/AIDS at home and referring appropriately for medical care; advocating the use of condoms; accepting family members with HIV/AIDS; and helping them to socialize and interact in the community.

We have covered implementation of PHC elements at family level; let us now discuss their implementation at location and divisional level.

Location and Divisional Level

- *Education*

As we discussed earlier individuals and communities can protect themselves against diseases and improve their health if they are well informed. Thus the role of the community here is to seek information and education from health care providers on how they can improve their health and also accept to change negative habits and customs which are harmful to their health.

- *Nutrition and Food Supply*

The activities at this level include: supervision of the CHWs by health committees; identification of high risk individuals and groups; providing relevant information to the District Development Committees (DDCs); and providing food security, promoting better food production, storage and marketing.

- *Water and sanitation*

The role of the community in the implementation of this element is to work closely with the Public Health Technician to protect and improve sources of clean water. They should also promote the

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construction and use of VIP latrines in the community and identify leaders to represent them in village health committees where issues of water sanitation are discussed.

- *Maternal, Child Health and Family Planning*

Since health workers are members of the local communities, they should train, support and supervise traditional birth attendants who manage pregnancy and labour in most rural communities. They should ensure availability of contraceptives and create awareness on the need to fully utilise the MCH/FP services available in the community.

- *Immunization*

The role of the community in the implementation of this element is to ensure that they take all the children for immunisation. Health workers at the community level should ensure that there is a constant supply of vaccines and that the cold chain is well maintained. Immunization should be provided on a daily basis at all health service delivery points and should be integrated with other MCH/FP services. Health workers should also train and supervise CHWs and provide outreach and mobile services where there are no statistic facilities.

- *Control of endemic diseases (malaria)*

The role of the dispensary and health centre is to support the malaria control activities at the community level. The type of support given depends on the local situation and the objectives of the malaria control strategy.

The following are the activities we undertake to control endemic diseases at the community level:

- Training supervision and follow up of CHWs and any other field staff;
- Ensuring constant supply of drugs and other supplies required by the community;
- Keeping records of clinical cases, parasitological cases, treatments and results of treatment given at this level;
- Compiling reports from CHWs/TBAs and providing them with feedback;
- Passing on information to the next level on frequency of malaria diagnosis and clinical results of treatment;
- House alternative drugs for those who need it.
- Setting out the criterion for referral and how to deal with emergencies due to malaria.

- *Treatment of Common Condition*

The role of the CHWs should be strengthened so that they can diagnose these conditions and refer or treat them early before complications set in. The health centre or dispensary should train and monitor CHWs; maintain records and reports of activities as well as forward them to higher levels. They should also ensure that field staff, including shop keepers, are well informed and equipped with necessary supplies such as ORS.

- *Essential drugs*

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At the community level, the health centres are responsible for technical supervision of the use of drugs by CHW. They should train the community on rational use of drugs.

- *Dental health*

At this level, the role of health workers is providing health education to the other community members on good dental health practices. Dispensary and health centre staff together with CHWs may need training on simple procedures for treating dental diseases.

- *Community Based Rehabilitation*

At the community level health workers have been trained to cater for CBR services within PHC. Their role here is to mobilise the community to adopt measures that promote good physical health and accept people with disabilities. They should also train CHWs on how to identify and prevent disabilities; basic techniques of disability rehabilitation at the community level; referral methods and integration techniques of the disabled;

- *HIV/AIDS, TB/STI Control*

At the community level health workers should facilitate health promotion activities; carry out outreach services including immunization and distribution of condoms; provide the correct treatment using national guidelines; promote early diagnosis and treatment; support home based care givers; and implement the HIV/AIDS and TB package

The District Level

The district focus for rural development strategy was an important policy shift in support of PHC. It provided new grounds for practical collaboration between the Government sectors and NGOs based at the district level. Before we discuss the implementation of the elements at the district level do the following activity.

The District-Level Health System

A health system where the central authorities within the Ministry of Health (MOH) are responsible for running the health services for the entire nation is known as a *centralised health system*. Primary health care is best implemented in a *decentralised* system, which transfers the authority and responsibility for planning, managing resources and/or decision-making from the central MOH to the district and local levels. Transferring management functions closer to the local health authorities gives the local communities a louder voice in determining how clinics and hospitals can improve the quality of health care being provided. You will learn more about how district health services are organised in Unit 4. Here let us look at the key PHC issues that are addressed at the district level.

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These include:

1. Co-ordination: the district is responsible for organising and coordinating comprehensive PHC services for the entire population. However, this coordination depends on adequate logistical financial support and training from the central authorities.

2. Health Management Teams: these facilitate shared decision making among the various levels of the health system up to the community level. Health Management Teams, such as the district health management teams, health centre management team, community health committees play an important role in ensuring that PHC services reach the people. Local authorities from other health-related sectors, representatives from NGOs and other interested groups may be included in these health management teams. Each team should be given advisory roles and regulatory powers for managing the PHC services (immunisation, maternal health/pre-natal care, water and sanitation, treatment of tuberculosis/leprosy, clinical services).

3. Community Participation
Community participation may be interpreted in various ways. It may range from district authorities informing community leaders about what the health sector has planned to community leaders being actively involved in making decisions (e.g., determining health priorities or strategies). The level of participation may greatly depend on the community leaders in local health committees: how they were selected, their capacity to mobilise community action and to demand accountability, and the amount of social and political support they can rally. The community should be encouraged to join forces with other sectors, organisations and groups when planning comprehensive PHC programs.

4. Resources for PHC
Implementing PHC requires resources to be readily available, particularly at the health centre level. Adequate and stable levels of staffing and essential supplies need to be maintained. In addition, district health authorities should encourage all levels to make maximum use of resources available locally. Sometimes these resources are not available because of logistical, financial, or managerial problems. In such situations, appeals for funding may be sent to donor's that are interested in strengthening the district health system infrastructure. Otherwise, local NGOs and existing community groups may be supported to extend services to outlying areas.

The Provincial/National Level

The provincial/national level is the highest level in the hierarchy of PHC implementation. The responsibility here is shared between the provincial teams and national programme managers. This is where policies are translated into strategies for implementation in the other levels we have mentioned. It is also where the major referral hospitals are found. As such, the activities that they undertake in the implementation of each PHC elements tend to be similar.

2.5 Primary Health Care in Relief Programs

Primary health care in emergency relief programs aims to do the following:

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- Reduce morbidity and mortality rates of the displaced population to regional norms.
- Build on existing knowledge and skills of the displaced community to improve overall health.
- Link emergency relief to rehabilitation, reconstruction, and development by building the capacity of the affected population. This will make it possible to sustain resources.

Lessons in PHC Planning

Planning and implementing of PHC in relief programs can be a slow and challenging process. Reasons for this include centralised decision-making, administrative delays, lack of supervision, and insecure professional health workers. The following lessons have been learned over the years about how a PHC program should be planned:

1. PHC can be adapted to all types of situations, including complex emergencies, provided the long-term goals are clear.
2. Factors that may influence planning PHC in emergencies include the political support of the host country, the historical experience of the health care system, the capability of the affected community, and the presence of NGOs and donors.
3. Decentralised planning helps to make relief programs more relevant to the needs of the displaced population rather than responding to the wants of the leadership.
4. When setting priorities, encourage active support and communication with the displaced community. This will lead to a consensus. The methods used will depend on the existing political structure.

Establishing an Emergency PHC Program

The type of emergency health services set up depends on several factors, including:

- the health system of the host country
- the available resources
- the context of the disaster
- the health needs of the affected population

Providing hospital-based care alone is appropriate only where a displaced population is concentrated within a limited space and the facility is accessible to all (located near a road or at the centre of the camp)? Setting up a field hospital is only justified when access to a referral hospital for surgical and obstetric emergencies is difficult or delayed.

To ensure a more cost-effective and sustainable program, relief agencies should establish an emergency PHC program within the framework of a district-level health system. This program should be:

- Based on the policies, standards and treatment protocols of the host country and integrated within the national health system.

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- Functioning in a decentralised manner that reflects the community's identified health needs and priorities.
- Comprehensive, involving all components of the health and other health-related sectors).
- Having clearly defined decision-making authority and responsibility for each level.
- Balanced in terms of the distribution of resources between curative, preventive, and promotive health programs.
- Sharing health information and promoting co-operation between all levels of the health system, and with other sectors and the community.

Relief agencies should aim at strengthening the existing public health infra-structure (basic health facilities, community health network, the local referral system and water supply, disease control, etc.) and at limiting dependence on external resources.

To establish emergency PHC services, first set up the health centre to function as the focal point for all PHC services in the area and establish a network of CHWs to extend services into the community. Community participation and inter-sectoral teamwork should be promoted from the beginning. Peripheral health units or dispensaries may be set up later, if necessary. All levels of the PHC system, from the home and community level to the district hospital, should be provided with essential resources (for example, staff, equipment, drugs) and logistical support. This will ensure PHC services are equitable and increase access to care.

Emergency PHC services should be co-ordinated within a functioning referral system so that the lowest skilled workers with minimum training provide the appropriate care at lower levels of the PHC system. At the same time, these workers must screen for conditions that require referral to higher levels of the system for care by more skilled PHC workers. Supervision should be arranged for all levels, carried by supervisors from the next higher level of the health system. For example:

- One auxiliary nurse-midwife based at a peripheral maternity unit may supervise ten traditional birth attendants within the community;
- A nurse-midwife at the health centre can supervise the auxiliaries at peripheral health units;
- Senior health workers based at the first referral hospital may supervise health centre staff.

This approach to supervision will ensure that a larger number of people receive quality health care more efficiently than when all patients are required to see only the most highly trained health workers. Each level of health care should also form a health committee in order to be accountable to the communities they serve. In addition, training community health worker teams to report their findings to different levels of referral system can greatly promote the effectiveness of PHC services at the peripheral health units.

Division of Responsibilities

Implementing PHC for emergency situations requires the community and other sectors to be involved in decision-making and on-the-job training and supportive supervision to be organised for all levels of the emergency PHC system. A unified approach for making referrals to other sectors or levels within the PHC system can be developed in the following way:

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- all field workers understand the PHC system: their responsibilities, functions of neighbouring levels, and the procedures for co-operation.
- each sector sets its own targets for services in terms of quality and coverage to make the system more effective.
- collaboration within the referral system is promoted to maximise the use of resources and labour, and to provide the appropriate level of care.

Specific responsibilities that may be defined for each of the following are described below:

Central Level

For PHC to be effective, the central Ministry of Health must be committed to its role of coordinating the emergency health system, mobilising resources and encouraging district-level decision-making. The main functions at this level should include the following:

- Making policies on emergency PHC operations and drawing formal agreements or memoranda with relief organisations and other providers.
- Regulatory authority for monitoring the level and quality of emergency PHC services and supplies.
- Promote inter-sectoral co-operation and inter-agency collaboration within the defined geographical area.
- Restrict relief organisations from setting up emergency PHC programs without considering the overall health needs of the affected community, in order to avoid duplication of services.
- Give formal support in the training of emergency PHC service providers.

District Level

The function of the district level should include:

- Co-ordinating health services in all PHC facilities in the district (including the referral hospital), based on the local budget and available resources.
- Encouraging all sectors to work well together.
- Initiating dialogue in the community and promoting active community participation in planning the district-level health system.
- Ensuring that community health workers have enough support and supervision.
- Collecting, compiling, and regularly forwarding health information to the central Government.

If the administrative capacity of the district level health authorities is weak, relief organisations may strengthen it by providing on-the-job training in PHC principles, district-level management, information systems, supervision, and health-related support. Training should target all senior managers from the district health office, the implementing agency and other health-related sectors who need to broaden their skills.

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Relief Program Level
Every relief sector (e.g., food and nutrition, water and sanitation, basic health care, etc.) should organise on the-job training for its staff in the following:

- Comprehensive PHC using problem-solving techniques that emphasise integrating preventive and promotive health interventions with the hospital-based curative and rehabilitative care.
- Developing and reinforcing standard ways of delivering services in order to improve the quality of the service. Standard methods should be adapted to the local situation and regularly reviewed.
- The planning process so all staff members will understand the program goals and objectives, their roles and duties, and the available resources.

Relief Worker Level
In any emergency operation, field workers are recruited in order to implement the PHC program. However, they must change from being the major "providers of PHC services to becoming enablers. Many field workers may be unwilling to take over new responsibilities since they, like many health workers, are only trained to *deliver* services to the beneficiaries rather than to make decisions about the program. Therefore, field workers need training in the following:

- How to increase the community's awareness of the association between poor health and poor living conditions or unhealthy behaviour. This will help strengthen active community support for multi-sectoral actions.
- How to meet regularly with the community to build support for the PHC program and to strengthen community participation.
- How to involve community representatives in determining priorities and in planning, implementing, and monitoring relief programs.

It is very important to recruit staff from among the displaced community. Recruiting an adequate number of female CHWs will increase access to individuals and households with the greatest need
.
Community Level
A partnership should be forged involving everyone who can improve the well-being of the community. This includes social groups, community groups, and traditional practitioners.
Getting communities to actively participate in decision-making on the emergency PHC program takes time and effort. They need to learn how to identify health priorities and the importance of co-operating and participating in PHC activities to improve their overall health.

The most important role of the community is to give regular feedback to the relief agency about the delivery of PHC services in terms of the following:

- equity in how services are provided
- access to care
- relevance between the services offered and the needs of the affected population

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In some situations, the local power structure may have to be readjusted to ensure satisfactory community participation. This can be achieved by including members from different social groups, such as women, youth, traditional healers, and school teachers in the health committees. This will ensure that the interests of the displaced population will be represented.

Components of Primary Health Care Emergency Health Care Priorities

- Promoting good nutrition : Provision of food rations and selective feeding programs
- Access to safe water and basic sanitation :Access to potable water and waste disposal systems
- Protection against cold (shelter, blanket, clothes)
- Improving maternal and child health care, including family planning
- Maternal and child clinics; later reproductive health programs with family planning
- Immunising against major infectious diseases
- Immunisation against measles, sometimes meningitis
- Preventing and controlling locally endemic diseases
- Control of communicable disease outbreaks including control of vectors and surveillance
- Fostering education on common health problems, their prevention, and control measures
- Health education based on a community health workers program
- Treating common diseases and injuries
- First level health services and a referral system
- Access to essential drugs; Provision of essential drugs

Emergency health care is built on the principles of PHC. Because resources for health care are always limited, both strategies reflect the basic needs approach toward addressing the priority health problems of a population. As a result, benefits of both strategies become clear when a large fraction of the total population has access to *comprehensive* health services on a regular basis (curative, preventive and promotive), and when those most in need are cared for.

2.6 Community Financing In PHC

The Bamako Initiative

The Bamako Initiative was conceived at the meeting of African Health Ministers, WHO regional committee and UNICEF held in Bamako, Mali in 1987.

It aim was to strengthen community action for improving the survival and quality of life of the communities and especially life of women and children in particular. It was concerned with resource mobilization at the community level in support of the PHC development and was therefore viewed as a major strategy for strengthening of PHC.

In Kenya, the government encouraged the setting up of community pharmacies run by community health workers (CHWs). The pharmacies stocked between nine and twelve essential drugs, which were charged at prices which not only covered their cost but also included a profit kept by the CHWs.

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In addition, insecticide-treated mosquito nets were sold at subsidised prices. Local people were positive and felt that prices were fair, though many struggled to find the necessary funds. CHWs were also positive because they could earn a small income – but there were dangers in this, particularly with the over-prescribing of unnecessary drugs to earn more money.

The funds were also used for recurrent expenditures like transport, fuel for supervising visits, allowances (tokens) for Village Health Committees (VHC) and CHW'S. This community-based financing mechanism was meant to ensure long term goal of self-reliance and sustainability for health care in overall context of socio-economic development. Local policies on user charges took into account issues of equity and affordability so that the poorest could also benefit.

Communities were initially provided with a stock of commonly used drugs and supplies. Through the use of community shops ("pharmacies") as outlets, the drugs and supplies were sold to the community members at a subsidized rate. The profit made from the sales was used to support PHC activities. The main donors of the Bamako Initaitive were WHO, UNICEF

Goal of Bamako Initiative

The Goal of the Initiative was to achieve universal accessibility of PHC by individuals, families and communities with emphasis on women and children.

Objectives of Bamako Initiative

- To strengthen the districts capacity for the management of PHC activities;
- To strengthen the development of essential projects at the community level;
- To strengthen the community's capacity to finance, organize and implement health activities and development at the community level.

The village health committee was supposed to be trained on book-keeping, budgeting and resource allocation. Community health workers and community pharmacists were trained in the manning of community shops/pharmacies. In addition, they were trained in the following:

- Curative care dispensing of medications for diseases like malaria, diarrhea, worms, conjuctivitis, scabies, anaemia and fever. Such drugs include Fansidar, paracetamol, aspirin, tetracycline eye ointment (TEO), FeSo4, Folic acid, mebendazole, and FP oral contraceptives.
- Home visiting in order to mobilize families towards better health habits including immunization of under fives.
- Growth monitoring of the under fives and referral of children showing growth failure to nearest health facility.
- Counseling on home improvement, use of latrines and safe disposal of household waste. Counseling on FP practices and community distribution of contraceptives including condoms.

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- Promotion of kitchen gardens to provide additional food variety to the family diet.
- Record keeping –
    - Registration of births and deaths in the community.
    - Curative records.
    - Record of money generated from sale of drugs.

The rationale of Bamako Initiative

- Lack of drugs – A large number of childhood deaths due to malaria, ARI, diarrhea occurred due to the fact that appropriate essential drugs were unavailable or inaccessible health institutions.
- The incidence and prevalence of top leading morbidity in Kenya i.e. malaria, RTI, skin conditions, worms, diarrhoea diseases and anaemia could be effectively reduced through curative, promotive and preventive health care measures at the community level.
- The government operates health services which attract heavy patient/client load with inadequate staffing. The adoption of B.I. reduced patient/client load in health facilities (institutions).
- The fact that communities could pull resources together and open own bank account cultivated a high sense of cohesiveness, ownership and common identity which was critical for community health action.
- Effective community health action reduced patient/client load in health facilities hence allowing the formal health care providers to concentrate on more complex health problems which they were trained to handle.
- Time sharing – the time wasted in search of drugs and transport costs to health institutions further justified the Bamako Initiative model.

The Bamako Initiative program was implemented differently in different countries with notable successes. However a decade later, two research initiatives conducted in Mali and Uganda, and later in Burkina Faso, concluded that:

- this policy did very little to improve or increase access to health care among the most deprived and excluded vulnerable population groups,
- this policy only served to marginalize certain population groups already disenfranchised due to the emphasis on financial sustainability and viability of health care organizations;
- the exemption mechanisms for alleviating the burden of payment and financial barriers for the poorest represent a technically feasible solution, while one not socially advocated.

2.7 Challenges faced in PHC implementation

Although considerable progress has been made in the implementation of PHC, in Kenya, the government has continued to face a number of challenges. Large gaps may be found in the planning and implementing of PHC and these are the challenges we shall now look at.

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- The major causes of morbidity and mortality in Kenya still remains diseases and conditions that can be easily prevented through immunization, improved personal hygiene and environmental manipulation. For example, it has been documented that 36% of the under-five population die before their fifth birthday due to preventable diseases and conditions. Diarrhoea alone account for 12.8% of under-five deaths in Kenya, while 12.2% are due to malaria. According to Kenya Demographic and health surveys of 2003, one out of every nine children born in Kenya dies before attaining their first birthday, while 40% of infant deaths occur during the first month of life.

- Curative services remain an expensive aspect of Kenya's health care delivery accounting for about 70% of health budget. Most of these funds are held up in tertiary and secondary level facilities, which are mainly located in urban areas. This situation has tended to impact negatively on the allocation of resources thereby undermining the principle of equity in health delivery.

- There has been an increase in the burden of diseases due to emerging and re-emerging diseases, as well as natural and human disasters. AIDS related illnesses such as pneumonia and TB have a national prevalence of nearly 10% and account for 14.2% of the burden of disease measured in terms of life years lost.

- Studies have shown that 55% of Kenyans lack access to safe water and sanitation, a situation that puts the population at risk of contracting diarrhoea and other communicable diseases. Air pollution, poor waste management and poor food control measures have also been on the increase.

- Malaria and respiratory diseases combined account for almost 50% of all reported diagnosis in public health facilities with diarrhoea increasing this to almost 60%.

- Maintenance of the present level of coverage achieved by many PHC programmes, such as KEPI, has remained highly dependent on continued support from donors, thus raising concern about their sustainability;

- Despite the seven-tier system in health care delivery, there are no clear guidelines on the referral procedures from one level to the other.

Other challenges include:

- Improper translation of PHC as *primary level of care (*first level health care in the pyramid), which ignores the overall integrated nature of PHC.
- The community may not be willing to take responsibility for the health care system.
- Drugs may not be available at lower levels of the PHC system. Therefore, patients will go directly to hospitals;
- Prolonged delays in health worker salaries may result in hostile attitudes towards patients;
- Referral system may not be functioning well;

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- Lack of supervision and training may result in poor quality of services;
- Different sectors may not be used to working together.

These challenges have raised a lot of concern not only within the ministry, but also among other key stakeholders in the health sector. To address these concerns, the ministry of health held a series of consultative meetings and workshops to try and reverse the deteriorating health situation in the country. A major outcome of this consultative process was the National Health Sector Strategic Plan (NHSSP). This plan was developed to address the constraints in the health sector and to adopt a sector-wide approach in their resolution. It was in response to the daunting challenge of operationalizing the 1994 Health Policy Framework Paper that the
Ministry of Health with her development partners developed the National Health Sector Strategic Plan NHSSP (1999-2004) and set up the Health Sector Reform Secretariat to spearhead the reform process.

Priority Health Packages In Kenya

The Ministry of Health also developed the strategy called the National Priority Health Package. This package ranks the health problems in Kenya according to the associated morbidity and mortality as follows;

High priority package

- Malaria prevention and treatment package.
- Reproductive health package.
- HIV/AIDS, TB prevention package.
- Integrated Management of Childhood Illnesses.
- Expanded Programme on Immunization.
- Control and prevention of major environmental health related communicable diseases such as cholera, typhoid fever and dysentery.

Middle priority health package

- Non-communicable diseases (Diabetes mellitus, Cadivascular diseases, Rheumatic fever).
- Reproductive cancers (Ca cervix, breast, scrotum, prostate gland).
- Mental health, drug and substance abuse.
- Injuries and accidents.
- Control of other vector borne diseases e.g. Leishmaniasis (kalaazar), Schistosomiasis, Elephantiasis.

Low priority health package

- Eye infections.
- Ear infections
- Worm infestation
- Other diseases not captured under the packages.

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The current NHSSP II (2005-2010) is formulated with the aim to reverse the downward trends in health indicators observed during the implementation of NHSSP I (1999-2004), applying the lessons learned and searching for innovative solutions. Its goal is to contribute to the reduction of health inequalities and to reverse the downward trend in the impact and outcome indicators. These health inequalities exist between urban and rural populations, between districts and provinces. They are related to gender, education and disability. The goal to reduce health inequalities can only be achieved effectively by involving the population itself in decisions on priority setting and consequently in the allocation of the resources. This requires a fundamental change in the existing governance structures in order to allow such a community ownership to take place.

Design Principles of NHSSP II

The design principles of NHSSP II can be summarised as follows:

1. Shift from disease burden (curative and vertical) to the human capital development approach that focuses on health promotion and provides comprehensive support to the various life cycles all Kenyans will experience. At the same time stimulate a Human Rights Approach in all interventions;
2. Scale up interventions by re-orienting the emphasis from facility-based (curative) services to increased preventive and promotive community-based care through (i) strengthening and expanding the role of Community Owned Resource Persons (CORPs) and health workers, working at grassroots levels; and (ii) removal of geographical and financial barriers (user fees);
3. Strengthen ownership and community oversight by (i) defining ownership of service delivery at district and health facility levels, strengthening district and hospital boards and health facility committees; (ii) jointly developing District Health Plans; (iii) supporting District Health Stakeholder Forum to monitor performance and (iv) improving information and responsiveness to claims (patient charters, publication of fee levels etc);
4. Reinforce the role of the MOH through (i) expanding geographical and financial access to services and at the same time improving their quality; (ii) reinforcing commitment to health as a human right; and (iii) improving resource allocation and financial targeting to underserved and poor areas; (iv) introducing regulative measures to improve collaboration.
5. Enhance collaboration to improve services in both the public and private sectors through (i) joint planning and (ii) a common monitoring system; (iii) sharing of resources like information, technology, training and finance; and (iv) providing incentives for improved collaboration (e.g. support the private sector to provide comprehensive care, immunisation services, use of DOTS).
6. Initiate steps to arrive at a SWAp: (i) elaborate a shared sector-wide strategy agreed by all stakeholders; (ii) elaborate jointly the Common Management Arrangements, necessary for implementing and managing the sector strategy; (iii) agree on new funding arrangements for the strategic plan and (iv) develop a Code of Conduct in which MOH and stakeholders establish a long-term commitment to collaborate in the implementation of NHSSP II.
7. Initiate a limited number of fast track activities to reverse the current downward trend and allow Kenya to catch-up and achieve (partially) the attainment of the health related MDGs. These fast-track activities are: (i) increase the number of health workers in the rural areas; (ii) initiate training of Community Owned Resource Persons (CORPS); (iii) scale up introduction of

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Insecticide Treated Nets (ITN) and (iv) distribute emergency delivery kits. The fast-track interventions will be reviewed and redefined annually.

8. Estimate the resources necessary to implement NHSSP II on the basis of well established unit costs for the various items of service delivery, taking the financial limitations, the MTEF ceiling and the criteria for priority setting.

The Kenyan Essential Package for Health (KEPH)

The Kenya Essential Package for Health (KEPH) focuses on the health needs of the individual through the six life cycles of human development. It represents the integration of all health programmes into a single package that focuses its interventions towards the improvement of health at different phases (cohorts) of the human development cycle. These phases are:

- Pregnancy and the New-Born (up to two weeks of age)
- Early Childhood (two weeks – 5 yrs)
- Late Childhood (6-12 years)
- Youth and Adolescence (13-24 years)
- Adult (25-59 yrs)
- The elderly person (60 yrs ++)

KEPH does not only define the target groups in terms of the life-cycle cohorts, it also defines where health services are to be provided. The preventive and curative services will be provided at six levels of care:

- Level 1, the community level, which is the foundation of the service delivery priorities;
- Levels 2 - 3 which receives KEPH activities related to promotive and preventive care, but also need various curative services.
- Levels 4 - 6 which undertakes mainly curative and rehabilitative activities of their service delivery package.

Review questions

1. Discuss the concepts and background of rural health, primary health care and community based health care.
2. What are the Principles, pillars and strategies of Primary Health Care?
3. What are the elements of Primary health Care and their implementation in Kenya;
4. Discuss how to organise PHC activities in relief programs;
5. Describe community financing in primary health care;
6. Discuss the challenges faced in PHC implementation.

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CHAPTER THREE

3.0 Behaviour Change and Communication

Learning objectives

By the end of this session, the student should be able to:

a) Explain the concept of behaviour change
b) Describe the behaviour change process
c) Describe various aspects of effective communication
d) Describe the various behaviours that impact on health
e) Develop messages for behaviours change and communication for the individual, family and community
f) Explain how to monitor and evaluate behaviour change.

3.1 Concept of Behaviour Change and Communication (BCC)

Communication is a two-way process of transmitting information from a source to a receiver with feedback. Good communication ensures the message is understood.

Behavior is anything that a person does. It

- Is a motivated action.
- Is deliberate, goal oriented?
- Is a conscious choice.
- Can be changed by choice.

3.1.1 What is BCC?

BCC is a process through which we strive to alter or influence people's attitudes (beliefs, feelings, action) and sustain the desired change.

It is the process of using communication approaches and tools to:

- Develop the skills and capabilities of communities to promote and manage their own health and development;
- Foster positive change in peoples' behaviour, as well as in their knowledge and attitudes;
- Work in partnership with families, schools, health services and communities to influence the social norms and policy environment within which they function.

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Behavior Change Communication (BCC) is a multi-level tool for promoting and sustaining risk-reducing behavior change in individuals and communities by distributing tailored health messages in a variety of communication channels. It is concerned with how organisms adopt in relation to their environment. Its focus is on how behavior is developed, maintained and can be altered. To understand the origin of behaviour we shall look at two theories of human behaviour.

3.2 Theories and Models of Behaviour Change

Theories tell us why people do what they do while Models tell us how they do it. The models are useful because they suggest specific areas for educational intervention.

There are quite a number of major theories of behaviour and behaviour change, some of which come from psychology. However, considerable attention has been given to models of individual behaviour change per se – but much less attention has been given to models or theories that attempt to understand behaviour change within groups and whole communities. The design of programs to reach communities requires an understanding of how those communities work, their barriers and enablers to change, and what influences their behaviours in general.

The theories of behaviour change can be divided into two main groups:

Stage Theories

These theories suggest that behaviour change occurs in stages or steps and that movement through these stages is neither unitary or linear, but rather, cyclical, involving a pattern of adoption, maintenance, relapse, and readoption over time. The work of Prochaska and DiClemente (1986) and their colleagues have formally identified the dynamics and structure of staged behaviour change. In attempting to explain these patterns of behaviour, Prochaska and DiClemente developed a transtheoretical model of behavioural change, which proposes that behaviour change occurs in five distinct stages through which people move in a cyclical or spiral pattern.

These are:

- Precontemplation: whereby the individual has no intent on the part of the individual to change his or her behaviour in the foreseeable future
- Contemplation: where people are aware that a problem exists and are seriously considering taking some action to address the problem
- Preparation: involves both intention to change and some behaviour, usually minor, and often meeting with limited success
- Action: where individuals actually modify their behaviour, experiences, or environment in order to overcome their problems or to meet their goals
- Maintenance: where people work to prevent relapse and consolidate the gains attained in the action stage. The stabilization of behaviour change and the avoidance of relapse are characteristic of the maintenance stage

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Further work undertaken and reported by Prochaska et al (1992) suggests that behaviour change can only take place in the context of an enabling or supportive environment. This supportive environment is made up of the following:

- SOCIAL FEATURES: nature of personal relationships; expectations of class, position, age, gender; access to knowledge, information.

- LEGAL FEATURES: laws determining what people can do and activities to encourage observance of those laws;

- CULTURAL FEATURES: the behaviours and attitudes considered acceptable in given contexts - eg. relating to sex, gender, drugs, leisure, participation
- POLITICAL FEATURES: systems of governance in which change will have to take place - can, for example, limit access to information and involvement in social action.

- ETHICAL & SPIRITUAL FEATURES: influence of personal and shared values and discussion about moral systems from which those are derived – can include rituals, religion and rights of passage.

- RESOURCE FEATURES: affect what is required to make things happen - covers human, financial and material resources; community knowledge and skills; and items for exchange

- CULTURAL FEATURES: - the behaviours and attitudes considered acceptable in given contexts - eg. relating to sex, gender, drugs, leisure, participation.

- POLITICAL FEATURES: - systems of governance in which change will have to take place - can, for example, limit access to information and involvement in social action.

- ETHICAL & SPIRITUAL FEATURES: - influence of personal and shared values and discussion about moral systems from which those are derived – can include rituals, religion and rights of passage.

- RESOURCE FEATURES: - affect what is required to make things happen - covers human, financial and material resources; community knowledge and skills; and items for exchange

Parallel with the work of Prochaska and DiClemente, Rogers, (1983) also developed a stage-based theory to explain how new ideas or innovations are disseminated and adopted at the community and population levels. Rogers identified five distinct stages in the process of diffusion of any new initiative or innovation. These are:

- knowledge,
- persuasion,

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- decision,
- implementation, and
- confirmation.

Rogers argued that the adoption of an idea is enhanced if a community or person perceives the idea as being superior to existing practice. That is, one has to see the relative advantage of adopting a new behaviour, such as washing hands after visiting the toilet. If a community does not see the advantage then they will not adopt the new behaviour. The new behaviour must also be highly compatible with the existing social system.

Social Cognitive Theory

Social Cognitive Theory explains human behaviour in terms of a dynamic and reciprocal model in which behaviour, personal factors, and environmental influences interact. The model believes that behavior is determined by expectations and incentives or the perceived value of an outcome. For example, people change behaviour because they can see some benefit such as improved health status or for the approval of others. Behaviour is also determined by beliefs about how environmental events are connected; opinions about the consequences of one's own actions; and expectations about one's own ability to perform the behavior needed to influence outcomes (self-efficacy).
Self-efficacy is one of the key concepts.

Self-efficacy refers to one's confidence in the ability to take action and persist in action. It is seen by Bandura (1986) as perhaps the single most important factor in promoting changes in behaviour. Measures of self-efficacy and some of the other key concepts from Social Cognitive Theory have also been identified as key determinants of movement through the stages of change, (Oldenburg, 1999).

Self-efficacy expectations have been found repeatedly to be important determinants of:

- the choice of activities in which people engage;
- how much energy they will expend on such activities and;
- the degree of persistence they demonstrate in the face of failure and/or
- adversity.

In general, higher levels of self-efficacy for a given activity are associated with higher participation in that activity.

Another theory that is similar to the Social Cognitive Theory is the Attribution Theory. This theory proposes that individuals generally view their performance (and thus, their successes and failures) as dependent upon ability, effort, task difficulty, and luck. In addition, causal influences are seen as either internal to the individual (e.g. personal ability), or external (e.g. barriers to community safety, lack of convenient and attractive travel alternatives).

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The distinction between internal and external attributions is an important one, in that how we attribute our personal successes and failures has been shown to be related to not only our behaviour, but our self-esteem, our perceptions of personal control, our self-efficacy for different tasks and/or performance situations, and our ongoing involvement in different activities.

How a person's attributes personal success and failure in a given situation, then, determines how they feel about the task, as well as the amount of effort they are likely to invest in the task the next time around. If a person thinks a task is too difficult and or they lack the ability to undertake it, then they tend to give up sooner, select easier alternatives, or even lower their goals. Conversely, when failure is attributed to external factors such as bad luck, individuals are likely to have higher motivations to continue and to try again for success.

The Health Belief Model

The health belief model advances the belief that indivual's behavior is guided by expectations of consequences of adopting new practices.

It has the following four concepts:

- *susceptibility*: does the person feel that they are at risk of getting a specific disease?
- *severity*: does one perceive that getting the disease has negative consequences?;
- *benefits minus costs:* what are the positive and negative effects of adopting a new practice? and
- *health motive:* does the individual have concern about the consequences of contracting the diseases?

3.3 Behaviour Change Process

Behaviour is determined by opportunity, motivation, sensory and motor capabilities and learning. What do they mean? Let us briefly look at each:

a) *Opportunity* - that means that the conditions/situation are favorable for that action to be done. For example, smoking is prohibited in all government premises therefore the smokers who work in these premises have to plan when and where to smoke.

b) *Motivation* - as seen earlier behaviour is controlled by positive or negative consequences. The positive consequences are those that produce pleasure and they serve to encourage the behaviour.

c) *Sensory and motor capability* - this simply means mental and physical abilities. We act what is on our mind.

d) *Learning* - to gain knowledge of a behaviour through instruction or experience. Whenever there is learning we see the evidence of change in behaviour. For learning

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to take place there are several steps to be followed because change of behaviour takes time and work.

The behaviour change process is a series of steps that involve:

Step 1: Creating Awareness

This involves pointing out the behaviour to the person in a way that is not judgmental. When you try to correct a person in an accusing or judgemental manner they become defensive and will end up ignoring the correction.

Be sensitive. Try to understand why the person engages in a particular behaviour. Use social influencing in creating awareness (To be discussed in the next section).

Appreciate the positive points about the behaviour and help the person to accept the negative points.

An example of a health worker talking to a cigarette smoker.

> *Health worker:*
> *I appreciate that cigarette smoking makes you to feel relieved of tension in your mind. Let us look at how this tension is relieved. The nicotine in tobacco gives you*
> *a feeling of excitement that makes you temporally forget your tension. After nicotine levels fall in your blood the tension is experienced again. Therefore you need to deal with the cause of the tension don't you think so…"*

Give as much information as you can concerning the behaviour. This gives the person a wider range of choices to consider when choosing what to do. This is called an informed choice making because the person will choose after considering the positive and negative aspects of their behaviour.

Step 2: Motivate To Change

Help the person see the advantage of changing their behaviour to a healthy one. Every effort made towards change should be acknowledged and encouraged. For example, if the person reduces the number of cigarettes, they used to smoke per day, congratulate and encourage them.

Do not force the person to change but let them see the need. Help the person also to strategize how they will begin the change then encourage them to carry on. Help the person to own the change because if they appreciate how it will benefit them they will work hard for it to succeed.

Step 3: Starting the Change

Help the person to begin to practice the new behaviour. Do not let the person wait to start another day, it should be started the moment they realise they need to change. This makes the resolution

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practical but if left to start later they may change their mind. Advice the person to begin slowly then progress on. Slow change or small change is easily sustained and improves or increases with time.

Support every effort made towards behaviour change and remind the person of the advantages of the change. Encourage the person to share their experiences in the change process and let them know what to expect, for example withdrawal symptoms among cigarette smokers or drug users. When a person knows what to expect they become more cooperative during challenges in the change process.

For you as a health worker to become an effective change agent, you need social skills or ways of convincing people to change their behaviours.

Step 4: Sustaining Behaviour Change

Once the person appreciates the benefits of the new behaviour they will be motivated to continue with it. Help the person to assess their progress in the change process. This helps them to appreciate that they are making progress, because if a person fails to see progress they give up easily. You should also encourage the person to help others in a similar situation. Once a person moves out to help others in mentorship or peer training it motivates them to sustain the change and become a good example.

Give the person skills to overcome relapses. These are:

- Avoiding old friends and social joints where the harmful habit used to be practiced
- Involving themselves in other activities when they think of old habits. This helps to keep the mind occupied with other things.
- Having another person to watch them and alert them the moment they go back on the old harmful habits (a chaperon).
- Doing away with all items that may lead to relapse, e.g., avoid alcohol in the house or cigarettes if your harmful habits involved them.

These steps are interactive in that you can go to and fro among them. For example, while helping to sustain behaviour change you create awareness on some issues that may have been tackled earlier. You also motivate in the process therefore the steps are not static but continuous. Help the person to own the change because if they appreciate how it will benefit them they will work hard for it to succeed.

Having learnt about the change process let us now look at the role of social influence in changing other people's behaviour. This will help us to become better agents in the behaviour change process.

3.4 Social Influence in Behaviour Change

Attempts by one or more people to change other person(s) behaviour or attitude are called social influence. This is achieved through:

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1. Persuasion Attitudes

Persuasion is the process of changing attitudes. In this process information is given to convince someone to change. Therefore, we can say that a source directs some message to some audience"

How do we persuade people to change their behaviour? The following approaches have been found to be useful and effective in persuading people to change:

- People who know what they are talking about, that is, those who have studied their subject well, tend to be more persuasive than those who do not know what they are talking about.

- Indirect messages are more successful in behaviour change than direct messages. This means that the message should not be spelt out as a rule or law. Many people would rather follow a suggestion other than a rule.

  An example of an indirect message is "Cigarette smoking is harmful to your health This message is thought provoking and can easily lead to change. An example of a direct message is Stop smoking. This message will provoke a reaction because the first question anyone would ask is "why?

  As you can see the intention of both messages is to stop people from smoking cigarettes but the way they are phrased can make a world of difference.

- People are often more susceptible to persuasion when they are involved in a dialogue rather than when they are lectured. Therefore you should not present your message in a lecture form, this will bore the audience. You should use different teaching methods to convey the message, such as, visual aids, drama, and video or radio lecture and audience participation in the communication process. This ensures that the audience remains alert and attentive to the message.

- A message is more effective if you adopt a two sided approach. This means that you should present both sides of the argument by supporting one side strongly while acknowledging the few good points of the other argument. Using our example of cigarette smoking, as a persuader you can acknowledge that tobacco business earns revenue to our government and the tobacco farmers earn their daily bread. However, remember that your goal is to stop cigarette smoking. So you should also present arguments that show how cigarette smoking affects the individual health and that of others. You can discuss issues such as addiction to nicotine, constriction of blood vessels, dangers of passive smoking, and so on. This approach enables the audience to accept your idea because they realize that you are broad minded. People don't like narrow minded arguments.

- Use messages that arouse strong emotions but not panic.

  For example:

  - Cigarette smoking kills

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- Cigarette smoking is harmful to your health
- Cigarette smoking is bad

The first example provokes fear and panic, the second one provoke thinking while the third one can easily be ignored

2. Conformity Pressure

People tend to change in order to conform or be like the majority of the people by following social norms. Social norms are classified into two namely.

a) *Descriptive norms*: these are based on what other people have done or would do in a given situation. They are more of a suggestion than a rule. They use words like may and can

b) *Injective norms:* these state what must be done in a given situation. These are rules because they are explicit and precise, for example, the written constitution or traffic signs. They use command words like should, must, ought and do. .

Social norms are intended for social order. It is therefore important to remind people of the social norms. This is done through making the audience think of themselves in terms of other people. For example:

- If you are in a group of people who do not smoke would you smoke in their midst?
- Would you want your parent to know that you are on addictive drugs?

These kinds of questions enable the audience to consider whether their actions are acceptable in the community. Since human beings are social beings, no one would like to be isolated. It is this need for social existence that the sender uses to pressurize the audience to conform to social norms that promote health.

3. Compliance Tactics

These are methods which you can use to get the audience to agree with your ideas or message.

Examples of compliance tactics include:

- *Ingratiation*: this is the most commonly used tactic where you make the audience to like you, by agreeing with them or making them see that you identify with them. However, you should not over do this tactic as it can have the opposite effect.

- *Commitment or consistency*: this is continuity of an issue. People like consistency and therefore you should focus on one goal at a time. Jumping from one issue to another shows indecision, unpreparedness and a lack of knowledge. You should start off with a generally simple request then later give a request that would ensure more involvement. If done tactfully the audience will easily be lured to comply to the ideas consistently.

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- *Tactics based on reciprocity*: reciprocity is a basic rule of social life. It simply means "treat others as you would like them to treat you". In this tactic you should start by making a big request, to the receiver or audience, which they would refuse. Then you scale it down by making a seemingly lesser request to which the receiver feels a subtle pressure to reciprocate by accepting. For example, let's use smoking to illustrated reciprocity. If you ask a smoker to stop smoking they can refuse. But if you ask them to reduce the number of cigarettes they smoke per day they may accept. The smoker accepts this because they consider that you do not disagree with them totally, so they decide to reciprocate by reducing the number of cigarettes they smoke. In the end, if the sender persists the habit is totally changed.

4. Obedience

As a health worker, you are considered an authority on health matters. The audience believes that you know what you are saying and so if you ask them to stop a bad habit they obey. However, as you may well know, it doesn't work in all matters. Sometimes this tactic provokes the receiver to ask a question like who are you to tell me what to do with my life?

In conclusion we take a quote from John Milton in his book *Paradise regained*, which says:

> *"It is more humane and more heavenly to use winning words to conquer willing hearts and make persuasion do the work of fear*

Fear used to be instilled in people to make them change behaviour but studies have revealed that fear provokes defiance. So try using persuation and winning words instead.

3.5 Behaviours That Impact on Health

Good health is a personal and collective responsibility. For the individual it involves developing a program of good health habits early in life and carrying them through adulthood and old age.

A health habit is a health related behaviour firmly established and often performed without awareness.

Health habit consist of behaviours like physical exercise, good eating habits, routine medical checkups, prompt health seeking behaviour, responsible sexual practices, personal hygiene, environmental hygiene, sleep, recreation activities and spiritual nourishment among others.

There is risky behaviour and health promoting behaviour. Health professional are to intervene and help people change the risky behaviors and practice health promoting behaviours. The health promoting behaviours are the habits undertaken by an individual to enhance or maintain their health. The risky behaviours are the habits that may expose an individual to a disease of ill health or cause a deterioration of an already existing health problem.

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The behaviours described above are health promoting behaviours. Risky health behaviours includes: smoking drinking alcohol, illicit sexual behaviour, careless driving, overworking and generally doing the opposite of the habits mentioned as health promoting.

Behaviours impact on health.

- Physical exercises - Enhance blood circulation and physical wellbeing of the individual
- Good night sleep – 6-8 hours sleep enables the body to rest. It also revitalizes the body thus enhancing performance.
- Balanced diet – helps the body to remain healthy reduces obesity and other nutritional disorders.
- Entertainment reduces stress levels. Even when work is too much we should get time to relax and laugh.
- Medical check ups – enables problem to be detected early and intervention carried out. It also helps in prevention of complications
- Spiritual nourishment – provides for our spiritual wellbeing and becomes our support in time of crisis.

Instilling good health habits and changing the bad ones is considered as primary prevention in that measures are taken to combat risk factors for ill health before an illness has developed. There are two general strategies for primary prevention:

- Employing behaviour change methods to get people to alter their problematic health behaviours;
- To keep people from developing poor health habits in the first place by educating them about health risks.

Practicing and changing health habits

Consider the following to be able to make an impact towards health promotion:

- Demographic factors, such as, education level, age available resources and stress level. Research has revealed that people who are under low stress levels and have a high social support system, practice better health habits than these under high levels of stress with few resources.
- Values and beliefs highly influence the practice of health habits.
- Personal commitment: that is the degree to which an individual perceives themselves to be in control of their health. Those who perceive themselves to be in control are more likely to practice good health habits than those who perceive their health as due to chance.
- Access to the health care delivery system for both services and education on health issues.

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- Social influence which might make an individual vulnerable to relapse to habits, for example, if a recovered alcoholic keeps frequenting the bar to chat with friends, he or she may relapse and start drinking again.

Thus as you can see, there are health promoting and compromising habits or behaviours. You need to educate people on health promoting behaviours and encouraged to continue with them. They also need to aware of the risks of health compromising behaviours.

Now that we have learnt about health promoting and risky behaviours we need to learn how to encourage health promoting behaviours. It is therefore important for us to look at how to develop messages for behaviour change.

Message Preparation

Presentations

A presentation is created in order to communicate ideas in a compelling and graphic manner. A presentation conveys more information than speech and it uses visual support. However, for it to be informative, persuasive and effective, it requires thorough preparation. You should consider the audience background, their knowledge of the subject education level or technical expertise and make adjustments to suit the audience. These are in addition to what we learnt in communication that the source should consider while sending a message.

You should also establish a clear central ideal goal/objective /purpose. Every element of the presentation should support the central ideal/objective. Therefore you should determine the following:

- What is to be done?
- How it will be done?
- What will be the significance?
- How do I expect the audience to respond?

Presentation delivery

To make an effective presentation, you should carefully prepare your thoughts and avoid reading the presentation word for word. Write notes in summary form and elaborate each point from the summary. Your presentation should have a structure based on your key ideas or objectives. This ensures a smooth flow of information.

Effective Visuals

Visual aids help to reinforce and expand the message as well as focus the audience's attention on the message. You should use visuals sparingly and avoid using them to entertain or in a way that distracts the audience from the central ideal. The acronym “PETAL” will help you to remember the characteristics of a good visual.

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P	*Pertinent* The materials developed should be relevant
E	*Engaging* format ⬌ choose a format that will involve audience by encouraging them to participate
T	*Timely* ⬌ be time conscious
A	*Appropriate* ⬌ ensure that the visuals are up to the standard of the audience and
L	*Legible* – ensure that the visual is large enough and clear enough to be seen by everyone.

Before you present a visual you should practice with the visual aid and other equipments before the actual presentation. To present it effectively, you need to use transition statements between visual. The acronym RDATwill helps you remember how to present a visual aid.

- Read the visual.
- Describe its meaning or significance
- Amplify it with an explanation or illustration
- Transition to the next visual

It is important for you to read the entire visual immediately it is disclosed and spend 1-3 minutes per visual aid.

Having looked at message preparation, let us now turn to the final section which discusses how to monitor and evaluate behaviour change.

Review questions

1. Describe the concept of behaviour change
2. Discuss the behaviour change process
3. What are the various aspects of effective communication
4. What are the various behaviours that impact on health
5. How can one monitor and evaluate behaviour change.

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CHAPTER FOUR

4.0 Community Diagnosis

Learning objectives

By the end of this session, the student should be able to:

a) Define the concept and purpose of community diagnosis;
b) Plan a community diagnosis survey;
c) Develop and pre-test tools for data collection;
d) Discuss how to execute the survey;
e) Write and disseminate a community diagnosis report and plan community action.

4.1 Concept and Purpose of Community Diagnosis

Community diagnosis is a process through which health workers together with members of the community identify the community's priority health problems, and together make plans of action and implement them. It points out where the health services should put their main efforts and resources. As you learnt in Unit 3 on Primary Health Care, a community's full participation depends on the four concepts of PHC or the 4A s.

These are acceptability, accessibility, affordability and availability using appropriate and local technologies. In the past, professionals including health workers always considered themselves experts on various issues. Health workers would visit a community and without consultation, impose certain sanctions or treatments without the community's consent. Also, some communities have received assistance particularly when there is a disaster and then being abandoned without being helped to address the causative factors. This has often resulted in dependency, lack of self-initiative to improve themselves or even a belief that they are helpless.

Luckily, this notion has changed. We now realize that people have the ability to help themselves if they are given some facilitation or guidance. People want to be independent and self-reliant. It is also true that people tend to cherish and care for what they have acquired through a struggle. Such involvement empowers individuals and makes them resourceful and self-reliant.

The community diagnosis concept therefore stresses that the community must identify its problems, prioritize them and draw a plan of action to address the identified problems. The community then implements this plan to resolve the problems. It emphasizes total community involvement. This is because the community knows its problems and priorities better than the health worker. When they actively participate in solving these issues they become bound by the decisions they make and feel motivated to see the plans through. Sometimes, one of the problems we experience as health workers is that communities may be more concerned about water or access to markets than they are about

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medical problems. In such situations, we need to look at all aspects of community development, that is, adopt an inter-sectoral approach, so that the real needs are tackled. There is no need to rush into a programme before there is understanding and commitment. This will just lead to failure. Sometimes you may even have to show your willingness and interest in what the community wants before you can concentrate on the main health problems.

As health workers, we talk all the time about patient diagnosis. Is this the same as community diagnosis? Let us look at the difference between patient diagnosis and community diagnosis.

4.1.1 Patient Diagnosis versus Community Diagnosis

Pertinent steps:

- Greet and welcome the patient and offer him/her a seat
- Ask for the patient's name, age, sex, marital status and patient's residential address;
- Take history and include details of the patients progress so far;
- Perform a physical examination;
- Carry out or request for special investigations;
- Make a differential diagnosis, followed by a specific diagnosis once results of investigations are confirmed. You may even state the expected outcome;
- Prescribe the most appropriate treatment;
- Give the patient a date when they should return for review. If the patient has an acute condition then you should hospitalize him/her for closer monitoring and review. The diagnosis of the patient will change depending on the presenting signs and symptoms each time he/she is reviewed;
- As the patient improves discharge him/her. You must remember that people are individuals, no two people are alike. Some may have started treating themselves before coming for your help.

In a nutshell, patient diagnosis can be in basic steps:

- Collecting basic information or history taking
- Planning programme or Diagnosis
- Implementation or treatment
- Follow-up and Evaluation

In community diagnosis, we follow the same basic steps as the ones we have seen in patient diagnosis. The only difference is that the amount of data is much greater and requires more lengthy analysis and processing.

In community diagnosis we start by collecting basic information. We collect information about the following:

- Local people and their environment;

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- The number of people and their distribution;
- The diseases the local people suffer from;
- The organisation of local health services.

Next we make a community diagnosis by identifying the main health problems and the reasons for them. We identify priority health problems and plan a community health programme (treatment) to solve these problems.

This is because health centres often have limited resources and many demands on those resources. There are simply not enough resources to solve all the health problems in the community. Therefore, the health care worker together with the community must select priorities for health action. Remember, it is important to choose only those problems that the community can do something practical about with the help of the local health team (health centre or dispensary staff).

The final step is follow-up to evaluate the programme and see if it has made the community healthier. You can evaluate an activity by counting or measuring things or simply by using your judgement. For example, to evaluate whether women are attending your antenatal clinic, you can check how many of the total number of pregnant women in your catchment area attend the clinic. You can also assess whether they are satisfied with the service by talking to women who attend the clinic and also to some who don't.

The tools we use in patient diagnosis are, for example, sphygmomanometer (BP machine) and stethoscope, weighing scales, thermometer, chairs and record books.

In community diagnosis, we use survey tools such as:

1. Maps;
2. Weighing scale;
3. Specimen bottles;
4. Questionnaires.

Having looked at the difference between patient diagnosis and community diagnosis, we shall now consider the purpose of conducting a community diagnosis.

4.1.2 Purpose of Community Diagnosis

The main purpose of carrying out a community diagnosis is to collect information on the following:

- Demographic data plus all the vital health statistics;
- Utilization of health services especially of maternal and child health clinics;
- The causes of morbidity and mortality (by age and sex);
- State of nutrition, diet, weaning patterns and the growth of pre-school and school going children;
- Patterns of leadership and communication within the community;
- State of mental health and common causes of stress;

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- State of the environment including water, housing and disease vectors;
- The community's knowledge, attitudes and practices (KAP) in relation to health related activities;
- Epidemiological details of endemic diseases;
- Available resources and services for overall development of health related activities i.e. education, agriculture, veterinary and social services;
- Socio-cultural and socio-economic class divisions within the community (social stratification).

Although a community diagnosis can yield a lot of data, it is not possible to collect it all in detail. Therefore, you need to tailor your survey to suit the specific information you want to collect.

4.1.3 Terminologies used in Community Diagnosis

Indicators are events or facts which can be measured to reflect the health status of an individual or community.

A variable is a characteristic within the study sample whose value changes among the study subjects. It is an observation made on the study subjects. Often two or more observations are made on a number of subjects. These observations either increase or decrease simultaneously or at varying levels. The two or more observations are called variables and their relationship is called correlation. For example, in a study of youths you may decide to observe their age and weight. In this case, the weight and age are the variables. There are two types of variables, namely, independent and dependent variables. In the example we have just given, age is an independent variable while weight is a dependent variable. Independent variables are plotted on the horizontal axis of a graph while a dependant variable is plotted on the vertical axis. Variables can also be described as qualitative or quantitative. Quantitative variables are expressed in numerical terms. e.g. age, height, area, weight and so on. Qualitative variables are adjectives that describe the subject of study, e.g., farmer, teacher, nurse, male, female, green, yellow, white, sadness, happiness, satisfaction, religion.

4.1.4 Ethical Considerations in Community Diagnosis

When conducting community diagnosis, it is very important to avoid taking any action that may be considered offensive by the community. You need to make sure that the tools you use to collect information are not in any way offensive to the community. They should not cause any physical, emotional, spiritual or cultural harm to that community.

I hope your answer included the following:

- Obtaining permission to enter into the community boundaries;
- Obtaining informed consent before interviewing clients, families or groups;
- Establishing rapport before exploring sensitive areas;
- Ensuring confidentiality of the data collected;
- Selecting good interviewers;
- Training interviewers.

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4.2 Planning a Community Diagnosis

The process of community diagnosis requires careful planning right from the beginning. It involves initial exploration and interaction with the community, planning of the survey, developing and pre-testing the survey tools and methods, and executing the survey and analysing the results. Once all the information has been gathered it must be documented and its conclusions disseminated to the community. The process of community diagnosis is made up of the following steps:

- Exploration
- Planning of the survey
- Developing and pre-testing survey tools;
- Execution of the survey and data analysis;
- Report writing, dissemination and community action.

4.2.1 Exploring the Community (Community Inventory)

Exploration simply means mapping out of a community in order to learn or discover about it. It is also known as community inventory. Ideally, we should only carry out a community diagnosis after a request by the community or the people involved in providing health care. The exploration phase is made up of three main activities:

- *Seeking permission and informing authorities in the community*

For the survey to succeed you must seek permission from community leaders. Where should you start? Think about it as you do the following activity.

You start by channelling your request through the official hierarchy of administrative leaders in that community. These are:

- Health personnel such as Medical Officer in Charge;
- Governmental officials such as Chiefs or DOs;
- Community leaders through, for example, the village health committee.

You should approach all these people, introduce yourself and clearly state the objectives of the survey and your plan of action. Remember, for them to give you permission to proceed they need to understand what you intend to do and how it will benefit the community. So be well prepared. The community leaders are extremely important to the success of the survey as they clearly understand what, how, when, and why things happen. They also influence other members of the community more directly than administrators who do not live in the community. You should therefore seek their assistance in implementing the survey. They will only cooperate if they perceive some beneficial results from their cooperation. So you should always plan a survey with the intention of carrying out

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an appropriate action programme for the community.

- *Seeking reactions of members of the community*

During this period of exploration, you also sound out the reactions of members of the community. This can be done by talking to people informally in market places and eating places. This way, you will easily find their opinions or problems in the community and their likely solutions. Also by talking to them you can gather information regarding possible resistance to the survey and how to approach different members of the community.

- *Gathering background data*

The period of exploration also presents you with the opportunity to gather background data about the community. For instance, the Medical Officer of Health in charge of the district will give you an overview of the health profile of the district. The District Commissioner will give you boundaries, the population and maps of the area. The District Education Officer will give information about educational activities and literacy levels of the community members. Other district heads will give information related to their areas of jurisdiction. Although some of the records might not contain the most recent data, you can make projections by estimating the current population figures in the community. Similarly by looking at other government reports you can gather information regarding the climate and weather conditions, water sources, and the road network. The older people in the community are a little-used resource and yet they can provide you with a lot of information on the community's history.

Gather information as you move around the community by:

- Questioning;
- Observing;
- Smelling;
- Listening.

4.2.2 Planning the Survey

This is a very important part of the process of community diagnosis. There is a popular adage which says "If you fail to plan you plan to fail". So if you fail to plan the survey carefully and properly, your study will be unsuccessful or will give you unreliable results.

During the planning phase, you should attempt to answer the following questions.

i. Why is the survey being done?

A survey is not carried out just to obtain interesting information. There must be good reasons and the reasons must be useful to the community. This question must therefore be answered to the satisfaction of all the leaders who are concerned with the community. If the community has

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given you permission to carry out the survey, it will expect and have a right to expect some beneficial results from its cooperation. You should therefore plan the survey with the intention of carrying out a proper action programme for the community. This information would most likely have been communicated during the exploratory phase. Once the community leaders understand the reasons and are ready to cooperate with you, a meeting of the members of the community should be called to explain why there is a survey, why they have been chosen, what will be involved, when it will be done and what will happen to the results. During this meeting you should invite government, health and community leaders so that the community can see who is supporting your work and who will be moving around their community and homes.

ii. Where will it take place?

You will have made this decision right at the beginning when exploring the community and seeking permission from various community and government leaders.

iii. Who will be interviewed?

It is usually not possible to interview everybody in a community unless of course it is very small. You will therefore need to select a sample from the total population which will be considered representative of what is happening in the entire population. The sample could be made up of individuals or households depending on the available resources and time. There are certain techniques used to choose a sample and we shall discuss them in detail later in this unit. It is also useful to talk to the local opinion leaders such as: the chief, village elders, members of organized groups such as church leaders and traditional healers, professionals in the area such as teachers and medical staff, and other extension workers.

iv. When will the survey take place?

If you intend to visit people at home, then I believe you said that you would avoid days when people are less likely to be at home, such as market days. It is important to choose carefully the days when the interviewers will be in the field in order to ensure that they find the people they want to interview. The exercise should also not coincide with seasons of important community activities such as planting, circumcision etc. Ideally you should decide when to conduct the survey after consulting the community members so that they are prepared for you.

v. What will be covered in the survey?

This will depend on what you want to learn about the community's health status and the information you have gathered during the exploratory phase. However, some topics like nutritional status of children under-five are often covered in a community diagnosis survey.

You will agree that each group of people has its own unique problems. It is important to find out what these problems are and then decide with the community which problems are the most important.

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Some of the specific areas that surveys address include:

- Screening people for diseases;
- Seeking to understand and identify ways of getting rid of negative cultural beliefs and behaviour that is causing ill health in the community
- Assessing the utilization of the available health services.

vi. Why are certain variables included and others left out?

Although it is your responsibility to determine what is included or omitted, it is very important to explain the reasons to leaders and members of the community. If you do not explain this clearly they may become disappointed and stop cooperating with you.

vii. What instruments will be used to measure the community's health status?

Usually questionnaires are used to cover most of the topics. However, in some instances, anthropometric measurements, physical examination and laboratory tests may also be necessary. We shall discuss questionnaires in more detail later in this section.

viii. How will data be collected and with what resources?

To answer this question you will need to specify the tasks that need to be done and then identify who will do them and how long it will take them. Consider the following factors:

- Time to travel to the study area;
- Time to locate the groups;
- Time and number of times each group will be visited. Allow time for following up defaulters;
- Calculate the number of interviews that can be completed in a day;
- Calculate the number of days that will be needed to complete the whole sample;
- Calculate the time needed for other parts of the study e.g., 5 days for preparation and pre-testing and 20 days for actual work.

The sign of a loud speaker on the left margin above would bring forth the information better if it was an illustration of a clock instead.

ix. How do we select and train the official interviewers?

Ideally, the interviewers should come from the community so that they are well known to its members. School teachers, school children, health centre staff, village elders and young educated people are some of the people who can help you to survey your area and fill in the questionnaires. However, if your interviewers are not from that community, then you will need to introduce them to the community leaders and if possible to the community members in a public meeting. Once the survey begins, they should wear identification badges and introduce themselves.

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Compare your answers with the following information.

The people you select for training as interviewers should have the following qualities:

- Be literate and well known to the community;
- Have the ability to display the right attitudes and opinions;
- Be able to explain the questionnaire effectively to the community;
- Be able to use the tools presented in your package;
- Be able to establish good rapport with individuals, families or groups they will meet;
- Be good listeners and sensitive towards other peoples feelings;
- Be able to relate well to the community members.

You should impress on the interviewers the value of working well with all sections of the community. If one of your tools addresses a specific group like the youths, you should select an interviewer of the same age or sex who can identify with the group. This helps to elicit the salient points from their responses. Whenever possible, select people who speak the language of the study group.

The people who are selected as interviewers have different educational backgrounds from yours and may interpret questions and answers differently. So you must train them on how to administer the survey tools. When training interviewers you need to explain the following:

- The purpose of the survey;
- How they should record the various expressions used by people to answer particular questions;
- What procedure they should follow to get cooperation from the people being surveyed.

If you intend to use a questionnaire you should go through it with them several times to ensure that they all have a common understanding of the questions and are able to ask them properly. They should understand the need to follow the questionnaire closely and in a standardized manner. If each interviewer asks questions in his/her own manner the answers will be unreliable because they may refer to different things. During the training you should hold mock interviews with the interviewers so that you can ensure that each one of them can handle the assignment. Use this opportunity to correct them and also to clarify issues about the questionnaire such as wrong translations and questions. Once you are confident that your interviewers can handle the job, you should carry out a trial test or pilot test on a section of the community who have similar characteristics as the study group. This gives them a feel of the real situation and helps you to assess them further. Remember that the pilot group should not be included in the study group.

I believe in your answer you said that they should do the following:

- Establish rapport by greeting the respondents and introducing themselves;
- Explain carefully why they have come and what is the purpose of the survey;

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- Ask if they are welcome to interview the family and if it is convenient for them at that time;
- Explain that they will be recording the information they collect;
- Emphasize that all information collected is confidential;
- Give them a chance to ask questions for clarification;

Having planned your survey and trained your interviewers, you now need to identify a representative sample which will answer your questions and provide you with the information and results you need. So let us now discuss sampling.

4.3 Developing and Pretesting Tools for Data Collection

Tools are implements that help us with our work. Before you embark on any procedure, you ensure that you have all the tools you need to do it effectively and that they are in the best possible condition. Similarly, before we embark on a community diagnosis survey we need to ensure that we have all the tools and instruments we need for measuring the community's health status.

a) The Questionnaire

Confirm your answer as you read the following discussion.

A questionnaire is a set of standardized questions designed to collect information about a specific aspect or issue in the community. It is therefore a tool for collecting information. Information from a questionnaire helps you to make plans for your health services and to evaluate them.

Before you design a questionnaire it is important for you to know what information you need to collect and how it will be used. It would help you to make a list of what you want to know. In other words, what do you really want to find out or achieve with the questionnaire?

i. What are the qualities of a good questionnaire?

A good questionnaire has the following qualities:

- Has simple and specific questions: avoids wording that is above the vocabulary or reading skills of the respondents;
- Has short and precise questions; the number of questions should not be too many or else they will put off the person being interviewed. In other words, keep it short and simple (KISS).
- Avoids use of abbreviations or jargon;
- Avoids questions that are too demanding and time consuming;
- Avoids bias in questions. Biased questions influence people to answer in a way that does not accurately reflect their position. For example, a question like "Do you agree with the majority of the people that health standards are falling*?"* implies that the respondent should agree.
- Avoids making assumptions. Questions such as "How many children do you have?" assume that the respondent has children. You should only ask this question after establishing the situation

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with the question "Do you have children?"

- Avoids double questions. For example, "Did the MCH talk help to identify ways to improve the sanitation and nutrition of your children?" It is better to ask about sanitation and nutrition separately.
- Ensures that your respondents have all the necessary information they need to answer your question. If they don't, provide them with a statement summarizing the background they need to know before they can answer the question.
- Has clear wording. Words such as majority, older people, regularly, might mean different things to different people and so should be avoided.
- Questions ask about simple common happenings;
- Questions range from known to unknown and from simple to complex.
- All the questions should relate to the purpose of the study. You should eliminate all "nice to know" questions, otherwise you will end up with information overload'
- Questions are acceptable to the people included in the survey. You should view the questions through the respondents eye and ask yourself the following:
  - will the question be seen as reasonable,
  - will it infringe on the respondents privacy,
  - will the respondent be able and willing to answer the question;
- Questions should not screen disease if no effective treatment can be offered for the cases found or if the condition is rare;
- Type of question should either be open or closed-ended;
- Questionnaire must be pre-tested before executing the survey. This helps to identify and eliminate questions that are defective or may lead to wrong information. You may even need to rephrase the questionnaire so that it can elicit the correct responses.

✓ Types of information

A questionnaire can help you collect four different types of information. These are:

- Information about what people know or how well they understand something i.e. knowledge. For example, *what is the major cause of accidental deaths among children in the home?*
- Information about people's beliefs, attitudes and opinions. Here you would be asking people to share with you their thoughts, feelings, ideas, judgment or their way of thinking. For example, *in your opinion does positive self-esteem prevent drug abuse among adolescents?*
- Information about people's behaviour. That is, what people have done in the past, present, and what they plan to do in the future. For example: *Have you ever attended an antenatal clinic?*
- Information about peoples attributes. That is, their personal or demographic characteristics, e.g. age, education, occupation, income, etc.

When you design a questionnaire you should be very clear about the objectives and type of information you desire to collect. Otherwise you may end up collecting peoples opinions when in actual fact you wanted to document their behaviour.

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- ✓ Type of questions

A questionnaire should be laid out in such a way that it provides easy flow from one topic to another. As we mentioned earlier it should have both open and closed-ended questions. They should be arranged in such a way that they allow natural flow of discussion.

An open-ended question is a type of question that allows the respondent to provide their own answer. It encourages the respondent to think and describe a situation in their own words. The respondent is not given any answers to select from. The answer given is best recorded in the respondent's own words. Although it is the easiest way to ask for information the responses are not easy to analyze. The answers are bound to be varied and so you need to categorize and summarize them.

Open-ended questions are useful because they give more information on:

- Facts and details which the researcher may not be familiar with;
- Opinions, attitudes and suggestions;
- Sensitive issues.

The following are examples of open-ended questions:

1. Describe exactly what the traditional birth attendants did when your labour started?
2. What do you think are the reasons for the high dropout rate of health committee members?
3. What would you do if you noticed that your daughter (a schoolgirl) has a relationship with her teacher?

As you can see, these questions require deeper thinking and provoke the respondent to elaborate when responding.

These are questions that offer the respondents a list of possible answers to choose from. They are specific and useful when you are interested in certain aspects of an issue. Although they produce more uniform answers than open-ended questions, they depend upon our knowing and including all the relevant answers in the list. Table 1 below gives three examples of closed-ended questions.

A good questionnaire should cover the following topics:

- measurement of the community's health status;
- Anthropometric measurements;
- Physical examination;
- Laboratory tests.

Once your questionnaire is ready, your next challenge will be to pre-test it. However, before we discuss how to pre-test your instruments, let us look at the other type of tool used in a community

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diagnosis survey, namely, focus group discussions.

b). Focus group discussions (FGDs)

This is a group discussion that gathers together people from similar backgrounds or experiences to discuss a specific topic of interest to the researcher. The group of participants are guided by a moderator (or group facilitator), who introduces topics for discussion and helps the group to participate in a lively and natural discussion amongst themselves.

A focus group is not a group interview where a moderator asks the group questions and participants individually provide answers. The focus group relies on group discussion and is especially successful where the participants are able to talk to each other about the topic of interest. This is important as it allows the participants the opportunity to disagree or agree with each other. It can provide insight into how a group thinks about an issue, about the range of opinions and ideas, and the inconsistencies and variation that exist in a particular community in terms of beliefs and their experiences and practices.

The discussion is usually "focused" on a particular area of interest. It does not usually cover a large range of issues, but allows you to explore one or two topics in greater detail.

- How can focus groups be used in a community diagnosis survey?

Focus groups can be used in the following ways:

i. Exploratory studies

Focus groups are a valuable method to explore a topic about which little is known, or little has been written in the past. For example, in order to set up a successful health education programme you need to understand people's traditional health beliefs. Focus groups can begin this process by providing the first in-depth descriptions of how the community sees the cause and treatment of certain illnesses. Focus groups can also be used to discover local terms used for signs and symptoms of illness, types of illness, and other concepts relating to health.

ii. Testing ideas about new programmes

In the planning phase of a new programme, it is possible to use focus group discussions to find out what the community feels about the new plan. You can use the method to see what the community identifies to be major problems or difficulties in existing programmes and incorporate their needs into the new programme. Focus groups can give you an understanding of how appropriate the new plan may be in terms of culture or technology.

iii. Solving specific programme problems

Sometimes we have programmes that have been running for some time and do not appear to be having the expected impact. You can use a focus group to explore such issues and identify the problems that may be hindering the success of the programmes.

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- How do we conduct a Focus Group Discussion?

In order to conduct a successful focus group discussion you should adopt the following 4 steps:

Step i. Preparation

First you recruit participants. Focus groups are "focused" because the participants usually share a common characteristic. This may be age, sex, educational background, religion, or something directly related to the topic. This encourages the group to speak more freely about the subject without fear of being judged by others who are thought to be superior. For example, young women may not be as forthcoming in their ideas and opinions in the presence of their mothers or mothers-in-law. The participants should have prior knowledge so that they can come prepared.

Step ii. Physical arrangement

It is good to make sitting arrangements that allow participants to see each other. Circular seating is the best as everybody can see each other. Avoid the traditional classroom type of sitting. Ensure the room is well lit and ventilated and has minimum or no disturbance so that the participants can concentrate on the discussion. The environment should promote talking and sharing.

Step iii. Preparing a discussion guide

You should prepare a set of questions that will help you to guide the discussion. These questions should also allow free flow from one aspect of the topic to the next in a relevant fashion. This helps the participants to think logically and build on the topic you are investigating. Have a mixture of general and more specific questions. If your questions are all general, you may not elicit detailed responses from the participants. On the other hand, if your questions are all specific, you may neglect to address and receive information on the "bigger picture." Make sure you have a variety of follow-up "probes" for each of your questions in the event that you need to clarify questions or have participants elaborate on their responses.

Step iv. The discussion

Before you proceed you should identify among your team one facilitator and one recorder and introduce them to the group.

The functions of the facilitator are to stimulate and support the discussion by:

- Introducing the topic and all the other participants. Self-introduction is better because it already sets in motion the tone of sharing for everybody.

- Reassuring them and explaining the purpose of the discussion and the type of information required. The participant also needs to know how and where the information will to be used.

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- Encouraging discussion by being enthusiastic, lively, humorous and showing interest in the group's ideas. The facilitator formulates and asks questions following the prepared guide. He/she should involve all the members but must remain neutral to all responses so that the participants can freely express their feelings, opinions and views.

- Encouraging involvement of all the members of the group. The facilitator should ask open-ended questions. He/she should identify and manage individuals who dominate the discussion and deny others a chance to respond. One way of managing domineering members is to ignore them and instead give attention to the other members when they want to give an opinion. Maintain eye contact with the shy ones and prompt them to talk by calling them by name and posing questions directly to them. Build rapport and empathize as necessary. You should also watch their expressions, mannerism and non-verbal communication. Try to understand what they are saying, communicating or insinuating. If something is not clear ask for elaboration. Avoid being an expert. The members may ask for your opinion. Re-direct the question back to them by asking for their opinion instead or what action they would take in respect of the question. After the session is over, you could share with them the information they were asking for. Control the discussion without blocking their freedom of expression and keep within the time allocated for this. At the end of the session you should thank them for sharing their time and ideas with you.

The recorder records all key issues raised in the session and other factors that may influence the interpretation of information in as much detail as possible. This involves noting down the responses from the group and observing and documenting any non-verbal messages that could indicate how a group is feeling about the topic under discussion. The following must also be recorded:

- Date, time, place;
- Names of participants;
- Description of the group level of participation including any dominant participant;
- Details of opinions of participants as much as possible using their own words especially for key statements;
- Details of emotional aspects and the vocabulary used. This will be particularly useful for developing questionnaires or health learning materials. If possible, a tape recorder should be used as well.

The recorder may also help the moderator if necessary. She or he may point out questions that are not well explored; questions missed, or suggest areas that could be investigated. The recorder should not be especially obvious to the group but should be able to communicate with the facilitator if required and help them resolve conflicting issues.

At the end of the discussion, the facilitator should seat with the recorder and review the discussion and complete the notes and evaluate how the discussion went. They should then prepare a full report of the discussion using the participant's own words. It is necessary to list the key statements, ideas and attitudes expressed during each topic. These statements are usually coded and written on the left hand margin while the comments are written on the right hand side. It may be necessary to formulate

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additional questions at this stage for those issues that were not yet clear or controversial.

Next, let us look at how we can pre-test our instruments in order to ensure that they are capable of collecting the data we need.

4.3.1 Pre-Testing the Instruments

It is very important to pre-test all the instruments you intend to use before they are finally administered. It enables the interviewing team to discern, alter or delete questions which are being misinterpreted or are too sensitive to be asked without offending people. It also gives you the opportunity to discover if the various parts of the questionnaire flow in a logical order.

According to Salant and Dillman (1994), any pre-test aims at answering the following questions:

- Does each question measure what it is intended to measure?
- Do respondents understand all the words?
- Are questions interpreted similarly by all respondents?
- Does each closed-ended question have an answer that applies to each respondent?
- Does the questionnaire create a positive impression – one that motivates people to answer it?
- Are the answers which respondents can choose from correct? Are some responses missing? Do some questions elicit un-interpretable answers?
- Does any part of the questionnaire suggest bias on the part of the researcher?
- Is the questionnaire too long?

Procedure for Pre-testing

During pre-testing, we examine individual questions as well as the whole questionnaire critically by:

- Asking colleagues to review the questions critically. This helps you to identify if the questions are clear and whether they meet the study objectives.
- Pre-testing the questionnaire on people who are very similar to your target group. It is also important to pre-test your instruments on a community that is very similar to the one in which the survey will be done.
- Simulating the actual data collection procedure. If for instance you are going to administer a questionnaire, you should give each interviewer/interpreter a copy and ask them to administer it to the group. Each interviewer should pre-test at least one complete questionnaire.
- Obtaining feedback about the form and content of the questionnaire. Were any questions misunderstood? Where the directions clear? Was the questionnaire too long or too difficult? How long did it take to fill it out? Was there enough space for the responses? We should leave in each questionnaire more space for answers than is planned for the final one. This gives the interviewer more space to fill in responses to questions which had not been anticipated.
- Checking if the questions produce the information we need.
- Trying out your tabulation and analysis procedure. Does the questionnaire yield data that can be

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analyzed in the way that is needed.

- Revising. Check the final draft by going over each question. Ask yourself what the information gathered from each question means and whether it will contribute to the study.

In this section we discussed how to develop and pre-test tools for a community diagnosis survey. We discussed two tools used for data collection, namely the questionnaire and focus group discussion. We also looked at the importance of pre-testing, what questions the pre-test should aim to answer and the procedure we should follow.

4.4 Execution of the Survey, Data Analysis and Presentation.

This is another important step in the process of community diagnosis. It requires just as much care as the planning stage. It involves going out to the field to collect information from the sample population you have selected. There are three stages involved in data collection. These are:

Stage 1: Interviewing the respondents

By the time we get to this stage, we have already developed the survey instruments, trained the interviewers on how to use these instruments and even pre-tested them. We do this in order to ensure that the correct standards are adhered to during the actual survey. Once the real survey begins, you should continue to work closely with the interviewers. Your presence reassures them and they also get the opportunity to clarify issues that may arise. On your part, you get assured that the interviewers are continuing to follow the techniques you taught them.

When the interviewer approaches a respondent, he or she should:

- Introduce him/herself by name;
- Show his/her identity card for the activity;
- Show the letter of permission to carry out the exercise;
- Explain why he/she has come and purpose of the survey;
- Establish rapport with the respondent so that they can feel at ease with each other ;
- Ask if it is convenient to interview the person at that time. If so, let the client/ family be comfortable and proceed. If not, ask for a more convenient time when this will be possible. This must be within the prescribed period of the exercise as indicated in the permit. In which case the interviewer must return punctually as agreed upon with the family or individual.
- Should the respondent refuse to co-operate, the interviewer should do his/her best to persuade such a respondent to agree. If the respondent remains adamant, the interviewer should politely thank the respondent for the time spent and proceed to the next interview. Sometimes people may refuse to respond to the question raised.

Compare your answer with the information given.

There are various reasons why people do not answer questions in a survey. These include among others:

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- If the people were not informed of the survey, its objectives and when it would be performed;
- If for some reason the person to be interviewed is temporarily away from home;
- Lack of interest in co-operating or active opposition to the survey.

In order to reduce the incidence of such opposition from your sample group, you should always make sure that the community is well informed about the study. If the target respondent is temporarily away from home, the interviewer should make arrangements to return when he or she is in. People who lack interest need a clearer explanation and persuasion so that they can see how they will benefit from the exercise. Activists can be difficult; if they remain adamant just politely thank them and let them free. They are just exercising their rights. If you find that a large number of people in your group are non-respondents, you will need to do a random selection from that group and re-approach them. The results you get from the new sample will be representative of the entire non-respondent group. Compare the results with the original sample of all respondents and calculate the difference.

Stage II: Data collection

Interviewers should be advised to use a pencil when filling out forms so that it is easier to make corrections. They should not erase a wrong response. If a mistake is made, the incorrect response should be crossed out and the correct response marked above it. Incorrect responses should not be erased because it is possible that if the erasure is incomplete, the response might not be legible or might be confused with a different response. The interviewer should fill in the responses at the time they are given. No response should be filled in afterwards because the interviewer may remember the response incorrectly. Ensure that every interviewer has all the tools they need to collect data such as tools for anthropometric measurements and laboratory specimen containers. You should avail a convenient carrier for these tools and check the packs daily to ensure that any specimens collected were handled correctly. It is good to remind the interviewers to re-check their measurements before they leave the client to avoid mistakes. They should also check the forms before they leave the respondent in order to be sure that all relevant information is satisfactory filled in the appropriate space provided. If the survey involves the collection of laboratory specimens, then you should make arrangements for their safe storage before they are transported to the appropriate place for analysis. Laboratory specimens need special care especially when handling, storing and transporting them because one lapse could cause the loss of an entire day's specimens.

At the end of the day, all the forms should be checked thoroughly by someone other than the interviewer. If data is missing it may be necessary for the interviewer to return the next day to collect it. At this stage it may be possible to begin tallying the results so that when the analysis begins all you have to do is add up the tallies instead of going through all the forms since the first interview. We shall discuss tallying later in this section.

We already answered this question in the sub-section on how to plan a survey. Can you remember what we said? Basically, we said that the best time to collect data is:

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- When the sample population will be available;
- When the team will be available;
- During an appropriate season when people are not too busy planting;
- On days other than public holidays and weekends when people are less likely to be at home.

During data collection, it is very important to ensure that there is quality control so that you do not end up with false or misleading conclusions.

I hope you said that the best way to ensure you get quality data is by:

- Avoiding biases when designing the questionnaire as we explained earlier;
- Providing an instruction sheet on how to ask certain questions and how to record answers;
- Selecting interviewers with care;
- Selecting and training the assistants carefully in all the procedures together with interviewers;
- Involving them in the pre-testing phase;
- Limiting the number of interviews that interviewers can conduct in a day so that they do not become too exhausted;
- Identifying assistants to carry out quality checks every day.

Stage III: Data Handling

This is the stage where you check data for completeness and organize it for analysis. The following guidelines will help you.

- Check to confirm that all the forms have been completed satisfactorily;
- Ensure that questionnaires are numbered;
- Identify one person to be responsible for storing data and specimens securely;
- Record forms should be sequenced and stored with clear labels.
- Make sure that all the information you need has been collected in a standard way;
- Develop an insight into the possible ways of analyzing data;
- Ensure availability of any resources needed for analysis, such as a computer;

Once you have collected the data, it is completely meaningless unless you can extract meaning through analysis. Next let us look at how to analyze the data.

4.5 Report Writing, Dissemination and Community Action

Often, after all the struggle of collecting and analysing data is over, we tend to feel like our job is complete. However, an important and time consuming part still lies ahead. That is, giving feedback, report writing and community action.

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Feedback means giving comments about how well or badly

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